Supreme Court, U.S.
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No. 97-53

In The

# Supreme Court of the United States

October Term, 1997

JANE M. ROBERTS, Guardian for Wanda Y. Johnson,

*Petitioner,*

vs.

GALEN OF VIRGINIA, INC., Formerly d/b/a Humana Hospital – University of Louisville,

*Respondent.*

**On Writ Of Certiorari To The United States Court Of Appeals For The Sixth Circuit**

## JOINT APPENDIX

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**ROBERTS *vs.* GALEN OF VIRGINIA, INC. (No. 97-53)**

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**Humana Hospital**
**University of Louisville**
**POLICY PROCEDURE**

No. -0418
Effective Date
8/ ___ /84
Page 1 of 2

Subject:

DISCHARGE CRITERIA
CRITICAL CARE COMPLEX

---

PURPOSE:

To establish guidelines for the transfer/discharge of patients from the Critical Care Complex.

POLICY STATEMENT:

Patients are discharged/transferred from the Critical Care Complex only with the order of a physician and when the need for intensive care or intermediate care no longer exists.

Prior to discharge/transfer patients will meet all of the following criteria:

1) Absence of Arterial Line
2) Absence of Pulmonary Artery Catheter
3) Absence of Intercranial Pressure Monitoring Device
4) Absence of life threatening dysrhythmias
5) Absence of severe Acid Base Imbalance (pH less than 7.2 or Greater than 7.6)

6) Absence of life threatening electrolyte imbalance

7) Absence of Acute respiratory failure. PA02 less than 60 (with or without supplemental oxygen therapy)

8) Absence of severe hypotension or hypertension – B/P stable – maintained without vaso active drips.

9) Absence of Blakemore Tube

In addition to the above, the Burn Unit also requires:

1) Wound reduced to 10% or less of total body surface area and free of active signs of wound infection

2) Patient or responsible person is able to carry out wound care and physical therapy as instructed (patient/responsible person has reviewed unit discharge booklet)

3) Patient is able to return to Clinics or arrangements have been made with other physicians for follow-up care.

Patients requiring an intermediate level of care are to be transferred to Transitional Care Unit. This includes all patients requiring: (See TCU Admission Policy #0417-E)

1) Cardiac monitoring

2) IV Antidysrhythmic drips

3) Tracheal suctioning every hour

4) Major treatments or dressing changes lasting 45" or more (2-3 times daily)

5) Supportive mechanical ventilation



6) Temporary cardiac pacemaker on standby.

7) V/S every two hours for more than 12 hours

Patients who no longer require the intermediate level of nursing care may be transferred to Medical/Surgical Units.

WP\P&P\CCCTraDis.P&P

Revised June 1991

Reviewed/Revised Critical Care Complex Policy Meeting 6/92.

**Humana Hospital**
**University of Louisville**
**POLICY PROCEDURE**

No. -0420
Effective Date 8/1/84
Page 1 of 3

Subject:

TRANSFER OF PATIENTS FROM THE CRITICAL CARE COMPLEX TO ANOTHER FACILITY

---

PURPOSE:

To provide guidelines for the safe and efficient transfer of patients from the critical care complex to another facility.

To assure that all patients who have an emergency medical condition are transferred in accordance with the Cobra requirements.

POLICY STATEMENT:

All patients who transfer from the Critical Care Complex to another facility and who are considered to have an emergency medical condition, will be transferred in accordance with the requirements established by COBRA.

Transfers will be made using good medical practice and for the benefit of the patient. No patient will be transferred arbitrarily. Transfers will be made for medical, not economic, reasons.

Transfers can take place only after the transferring physician has received approval for transfer by the patient/ patients' family and the accepting physician.



The Critical Care Complex will secure a written, informed consent prior to transfer. This consent must include the risks and benefits of the transfer.

When transferring a patient to another facility, the following must accompany the patient:

1. copies of all medical records related to the emergency condition and available at the time of transfer;

2. Copy of the patient's request for transfer or the physician's transfer certification; and,

3. The name and address of any on-call physician who refused or failed to appear within a reasonable amount of time to provide necessary stabilizing treatment.

All patient requests for transfer must be in writing and must include a statement of the Hospital's obligations and the risks of transfer.

Social Service will be contacted prior to the transfer of any patient from the Critical Care Complex to another facility. Social Service will make the ambulance arrangements.

Air ambulance transfers are arranged through Social Service, using only authorized medical air transport services who supply the medical flight personnel and the equipment/supplies.

ACTOR

Physician

ACTION

1. Determines need for transfer (patient/family request or medical necessity) and arranges physician acceptance of the patient at the receiving facility.

2. Writes transfer order in the patient's medical record.

3. Completes transfer summary and signs, indicating the date and time.

4. Explains risks and benefits of transfer to patient/family and completes the physician section of the "Physician Certificate and Patient Request for Transfer" Form including:

   A. name of patient
   B. name of receiving facility
   C. name of receiving physician
   D. summary of risks
   E. summary of benefits

ACTOR

Physician/Registered Nurse

ACTION

5. If the patient/family is requesting the transfer, completes the patient request section form including:

   a. name of patient
   b. name of receiving facility
   c. mode of transportation



   d. risks of transfer
   e. physician's name who discussed risks/benefits

6. Obtains patient's or nearest of kin's signature on form, along with the relationship to the patient, a witnesses signature, the date, and the time.

7. Accompanies patient during transport to the receiving facility, as patient's condition indicates.

   NOTE: All patient's who are being transferred to the intensive care unit at the receiving facility will be accompanied by a physician or a Registered nurse.

ACTOR

Registered Nurse

ACTION

8. Completes appropriate sections of transfer summary.

9. Assures discharge planning is documented.

ACTOR

Unit Secretary/RN/LPN

ACTION

10. Notifies Social Service of plan to transfer patient.

11. Places patient's name, medical record number and unit/bed on all forms pertaining to the patient's transfer using the addressograph.

12. Places the yellow copy of the Physician Certificate and Patient Request for Transfer form, the transfer summary, a copy of the physician's discharge notes and copies of any other approved, requested chart information in an envelope with the patient's name on it for transfer to the receiving facility.

13. Gather's [sic] the patient's personal possessions for transfer.

14. Prepares for transport by gathering supplies/equipment needed.

ACTOR

Registered Nurse

ACTION

15. Telephones receiving registered nurse and reports patient's condition and nursing needs.

16. Completes Nurses Notes, including:

    a. transfer summary
    b. vital signs
    c. general condition on departure
    d. time of departure
    e. method of transport

ACTOR

Registered Nurse or Unit Secretary

ACTION

17. Enters any charges on computer.

18. Informs physician of arrival of ambulance if physician to accompany patient.

19. transfers patient.

WP\P&P\CCCTranAnoFac.P&P

Reviewed/Revised in the CCC Policy Meeting on June, 1992

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**Humana Hospital University**
**POLICY PROCEDURE**

No. 768-0901
Effective Date 6/15/85
Page IX

Subject:

NURSING HOME PLACEMENT

---

POLICY:

Plans for Nursing Home Placement will be coordinated by Social Services with consultation and collaboration of the patient, his family and other medical disciplines.

PROCEDURE:

Actor:

Social Worker

Action:

1. Councils with patient, family and other "health care disciplines" regarding continuing care needs of the patient and the resources of the patient and family to meet these care needs (both physical and emotional).

   A.

   1. Home Health Care
   2. Nursing Home Placement

   A. The determination is made by the patient and family, that the care needs exceed their abilities and nursing home placement is requested.

2. Asks patient and family if they wish to suggest a licensed nursing facility that has the appropriate level of care needed.

3. Advises patient and family that the preferred facility will be contacted, but if there is no available bed, other licensed appropriate facilities will be contacted for admission.

Actor:

Physician

Action:

4. Documents the need for Nursing Home Placement.

5. Orders the following:

   A. Review for long term care

   B. CBC, CXR, VDRL, PPD

Actor:

Social Worker

Action:

6. Contacts appropriate facilities considering family request first and proximity. If the above facilities have no available bed, then other facilities are contacted until placement is found.

Actor:

Nursing/Physician

Action:

7. Assists Social Worker with nursing home placement as needed, relaying care needs to the worker and/or directly to the nursing home as requested.



Actor:

Social Worker

Action:

8. Notifies patient and family of accepting facility.

9. Advises patient and family of possible consequences if placement is refused.

Actor:

Patient/Family

Action:

10. Decide if they will accept the placement plan. If plan is refused, patient will be discharged in the care of himself and/or family.

Actor:

Social Worker

Action:

11. Communicates with physician and nursing, the discharge plan.

12. Makes referrals to a licensed certified home health agency if this is the decision of the family when placement is refused.

13. Refers patient to Adult Protective Services, if patient's discharge home appears to be a situation that may be harmful or neglectful.

Actor:

Patient/Family

Action:

14. Accept nursing home placement as agreed on.

Actor:

Social Worker

Action:

15. Asks physician for a tentative discharge date.

16. Informs family and receiving facility, the tentative discharge date.

17. Reports to nursing and physician, the date facility will accept patient.

Actor:

Physician

Action:

18. Dictates and signs a STAT discharge summary and completes physicians information part of Transfer Form.

Actor:

Social Worker

Action:

19. Completes Social Evaluation part of Transfer Form.

20. Makes appropriate transportation arrangements.

Actor:

Nursing

Action:

21. Completes Nursing Assessment of Transfer Form.

22. Checks to see if all requested medical papers are completed for transfer.

23. Prepares patient and his personal belongings for discharge.



Actor:

Social Worker

Action:

24. Sends appropriate paper work at time of transfer.

    a. History & Physical
    b. Chest x-ray
    c. VDRL
    d. CBC
    e. Signed Discharge Summary
    f. Transfer form
    g. PPD results
    h. Statement of emergency
    i. Decubitus cultures
    j. Open wound cultures

25. Make transportation arrangement as agreed upon by patient or family.

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**Humana Hospital**
**University of Louisville**
**POLICY PROCEDURE**

No. 768-0912
Effective Date 7/10/91
Page 1

Subject:

REHABILITATION REFERRALS

---

POLICY:

Plans for Rehabilitation placement will be coordinated by Social Services with consultation and collaboration of the patient, family and medical personnel.

PROCEDURE:

Actor:

Physician

Action:

1. Documents the need for rehabilitation consultation.
2. Orders the following when needed:
   A. PPD, VDRL, cultures of trach sites, open wounds, sputum, etc.

Actor:

Social Worker

Action:

3. Monitors progress of patient for appropriateness of rehab consult.
4. Discusses rehab with physician if physician hasn't already ordered rehab consult.



5. Discusses rehab process with patient and/or family.
6. Reviews patient's insurance coverage and refers for medicaid if eligible.
7. Contacts appropriate rehab facilities and/or contacts a physiatrist group to perform rehab evaluation.
8. Follows-up with rehab facilities to determine their acceptance or denial of patient.
9. Notifies family of which facilities are an option for patient.

Actor:

Patient/Family

Action:

10. Reviews options available and decides on a rehab facility.
11. Notifies social worker of their decision.
12. Fills out appropriate paperwork for the accepting facility.

Actor:

Physician

Action:

13. Notifies social worker of readiness for rehab transfer.
14. Dictates stat discharge summary and completes transfer form.

Actor:

Social Worker

Action:

15. Checks bed availability and arranges a time for transfer.

Actor:

Nurse

Action:

16. Assists with paperwork, completes nursing assessment on transfer form.

17. Calls report to accepting facility's nurse.

Actor:

Social Worker

Action:

18. Sends appropriate paperwork:

    A. H&P
    B. Culture reports
    C. Physical Therapy notes
    D. Speech notes (if appropriate)
    E. CT Scans (if needed)
    F. D/C Summary
    G. Transfer form

19. Notifies patient/family of plans for transfer.

20. Calls ambulance for transport.

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UNITED STATES DISTRICT COURT
WESTERN DISTRICT OF KENTUCKY
AT LOUISVILLE
CIVIL ACTION NO. C-93-0544L(CS)

[Caption Omitted In Printing]

* * *

DEPOSITION FOR PLAINTIFF

* * *

TRANSCRIPT OF DEPOSITION OF NANCY FRED, TAKEN, PURSUANT TO NOTICE, ON MONDAY, NOVEMBER, 22, 1993, AT 1:30 P.M., * * *

* * *

[p. 25] A. (READING.) "SPOKE WITH PATIENT'S AUNT, JANE ROBERTS, WHO IS UNABLE TO LOCATE PATIENT'S HUSBAND. HAVE FOLLOWED HIS INSTRUCTIONS AND PAGED HIM REPEATEDLY WITHOUT EFFECT. HAVE LOCATED NURSING HOME IN CHANDLER, INDIANA. THAT HAS CLINICALLY ACCEPTED PATIENT, PENDING THEIR COUNCIL ON AGING'S OKAY. WILL KNOW TODAY. HAVE INFORMED PATIENT'S MOTHER AND AUNT WE MAY TRANSFER PATIENT TO CHANDLER NURSING HOME IN NEXT DAY OR TWO. NEW MEDICO ALSO STILL INVOLVED; PREDICT APPROVAL WILL TAKE ANOTHER ONE TO TWO WEEKS."

Q. NOW, HAVE YOU EVER REFERRED A PATIENT TO THIS CHANDLER, INDIANA, NURSING FACILITY PRIOR TO THIS?

A. YES.

Q. SO YOU WERE FAMILIAR WITH THEIR FACILITY AND WHAT THEY HAD TO OFFER.

A. RIGHT, YES.

Q. AND HOW DID YOU COME INTO CONTACTING THEM OR REFERRING THEM TO HER? APPARENTLY, IT WASN'T ON YOUR FIRST LINE OF REFERRALS.

A. WELL, EVIDENTLY, SHE'S BEEN WEANED, YOU KNOW, FROM THE VENTILATOR, AND THAT MEANS THAT OTHER NURSING HOMES COULD HANDLE HER CARE. NOW, I CAN'T, I'M JUST SURE THAT HANOVER OBVIOUSLY COULDN'T TAKE HER ONE WAY OR THE OTHER AT THIS POINT, OR WE WOULD HAVE PROCEEDED WITH HANOVER. SINCE THAT WE KNEW, AS OF, YOU KNOW, THE INITIAL CONTACT WITH THE HUSBAND, WAS AN OKAY DISTANCE, AND YOU KNOW, FOR HIM AND [p. 26] EVERYTHING. SO I MEAN, IT JUST GOES WITHOUT SAYING, I MEAN, TO ME, MAYBE NOT TO YOU, THAT HANOVER WAS NOT AVAILABLE AT THIS POINT, AND NONE OF THE OTHER VENTILATOR-DEPENDENT NURSING HOMES HAD RESPONDED THAT THEY COULD TAKE HER AT THIS POINT.

Q. OKAY.

A. BUT THE PROBLEM WITH CHANDLER IS IT'S, YOU KNOW, IT WAS A LITTLE FARTHER, YOU KNOW, AND I'M SURE LESS DESIRABLE FOR PATIENTS IN LOUISVILLE AND FURTHER SOUTH.

Q. WHERE IS CHANDLER, INDIANA?

A. IT'S A SUBURB OF EVANSVILLE.



Q. LET'S GO TO THE NEXT DAY, WHICH IS JULY 21ST, AND "EXHIBIT G."

A. (READING.) AS OF THIS A.M., CHANDLER NURSING HOME IS PLANNING TO ADMIT PATIENT ON THE 22ND. THEY DO NEED TO OBTAIN PRIOR OKAY FROM THEIR COUNTY'S COUNCIL ON AGING THIS A.M. AWAIT FINAL DECISION. WILL NEED SIGNED EMERGENCY NURSING HOME ADMISSION, TRANSFER FORMS, INCLUDING ORDERS FROM PHYSICAL THERAPY AND SPEECH THERAPY. WILL NEED STAT D/C SUMMARY AND D/C ORDERS.

Q. ALL RIGHT. SO AT THIS POINT, IT LOOKS LIKE SHE'S GOING TO BE TRANSFERRED TO CHANDLER.

A. YES.

Q. LET'S GO TO THE NEXT DAY, WELL, ACTUALLY THE SAME DAY, ON "EXHIBIT H."

[p. 27] A. (READING.) "CHANDLER NURSING HOME HAS ACCEPTED PATIENT. COUNCIL ON AGING HAS OKAYED TRANSFER. PATIENT TO BE TRANSFERRED 7-22, HOPEFULLY BY 11 A.M. PATIENT'S HUSBAND WAS INFORMED AND AGREES. PATIENT INFORMED. PLEASE SIGN EMERGENCY NURSING HOME ADMISSION NOTE AND COMPLETE TRANSFER FORM. FRONT OF CHART AND DICTATE STAT D/C SUMMARY BY 9:30 TO 10 A.M.

Q. IT STILL LOOKS LIKE CHANDLER IS THE PLACEMENT AT THIS POINT, CORRECT?

A. YES.

Q. ON JULY 21ST, IT STILL APPEARED THAT CHANDLER NURSING HOME WAS THE PLACEMENT.

A. YES.

Q. IT SAYS "PATIENT INFORMED." WAS WANDA'S CONDITION SUCH AT THAT TIME THAT SHE WOULD HAVE UNDERSTOOD THAT? WHAT WOULD THAT MEAN?

A. YOU DIDN'T KNOW; YOU JUST DIDN'T KNOW. SHE, IT WAS THOUGHT THAT MAYBE NEUROLOGICALLY, SHE WAS STARTING TO UNDERSTAND A LITTLE BIT. IT WAS INCONSISTENT, AND SO –

Q. STILL COULDN'T TALK AT THIS POINT, CORRECT?

A. COULDN'T COMMUNICATE.

Q. STILL COULDN'T MOVE, I SUPPOSE.

A. HEAD NODS WERE INCONSISTENT, BUT SOMETIMES FORTHCOMING.

Q. SO THIS INDICATES THAT YOU TOLD HER –

[p. 28] A. TRIED, TRIED TO COMMUNICATE –

Q. SO IT INDICATES YOU ADVISED THE PATIENT. DO YOU HAVE ANY WAY OF KNOWING WHETHER SHE UNDERSTOOD THAT SHE HAD BEEN ADVISED SHE WAS BEING TRANSFERRED TO CHANDLER AT THIS POINT?

A. NOT REALLY.

Q. LET'S GO AHEAD TO THE NEXT DAY, WHICH WAS JULY 22ND, AND I BELIEVE "EXHIBIT I."



A. (READING.) "CHANDLER NURSING HOME DIRECTOR OF NURSING WILL SEE PATIENT AT OUR FACILITY PRIOR TO HER TRANSFER TODAY TO ASSESS ALL POSSIBLE TREATMENT NEEDS AND SEE THAT NECESSARY STAFF AND EQUIPMENT ARE IN PLACE. HAVE COPIED CHART TO SEND WITH PATIENT. WILL ORDER AMBULANCE AFTER CHANDLER DIRECTOR OF NURSING SEES PATIENT. PATIENT WILL BE PLACED AT SKILLED LEVEL."

Q. ALL RIGHT. NEXT PAGE, THE SAME DAY, I THINK IT'S "J." WOULD YOU READ THAT?

A. (READING.) "CHANDLER HEALTH CARE HAS NOW RECANTED PRIOR DECISION TO ACCEPT PATIENT. SOCIAL SERVICES DIRECTOR, GOLDIE MURPHY, HAS SPOKEN WITH CHANDLER'S ADMINISTRATOR. AND WE EXPECT FINAL DECISION THIS P.M. IN INTERIM, HAVE ASKED MAYFAIR NURSING HOME, INDIANAPOLIS, TO EVALUATE PATIENT. THEY DID SO THIS P.M. WILL ALSO HAVE SOUTHSIDE NURSING HOME, INDIANAPOLIS, EVALUATE PATIENT TOMORROW, IF NECESSARY. NEW MEDICO HEAD INJURY IS ALSO STILL INVOLVED. THEIR DECI- [p. 29] SION PROCESS TAKES LONGER, AND NO FINAL DECISION IS POSSIBLE FROM THEM FOR AT LEAST ANOTHER WEEK. ALSO, NEW MEDICO WOULD REQUIRE "G" TUBE, "G" OR "J" TUBE."

Q. ALL RIGHT. DO YOU HAVE ANY IDEA WHY CHANDLER HEALTH CARE RECANTED THEIR DECISION TO ACCEPT WANDA?

A. NO, I DON'T.

Q. THEY DIDN'T TELL YOU WHY THEY WEREN'T GOING TO TAKE HER?

A. THEY JUST SAID THEY DIDN'T THINK THAT THEY COULD MEET HER CARE NEEDS AT THAT TIME.

Q. WAS THIS THE DIRECTOR OF NURSING WHO YOU HAD REFERRED TO WAS GOING TO MAKE THE EVALUATION?

A. YES.

Q. AND DID THAT CONVERSATION BETWEEN YOU AND THAT DIRECTOR OF NURSING TAKE PLACE AFTER THE EVALUATION WAS DONE THAT DAY?

A. THE EVALUATION WAS DONE. AND I THINK THEN, SHE PROBABLY WENT BACK AND CONSULTED WITH, YOU KNOW, HER STAFF.

Q. DO YOU REMEMBER, DID SHE CALL YOU AND TELL YOU THIS, OR DID YOU SPEAK WITH HER AT THE HOSPITAL HERE IN LOUISVILLE?

A. I THINK THEY WERE AT THE HOSPITAL. THEY WERE FREQUENTLY AT THE HOSPITAL, CHANDLER, THE ADMINISTRATOR AND HIS DIRECTOR OF NURSING.

Q. AFTER CHANDLER HAD RECANTED AND DETERMINED THAT [p. 30] THEY COULDN'T MEET HER CARE NEEDS, AT THAT POINT, WERE YOU ABLE TO CONTACT ANY OF WANDA'S FAMILY TO ADVISE THEM THAT CHANDLER WAS NO LONGER IN THE PICTURE?



A. I JUST DON'T REMEMBER THE SPECIFICS OF WHAT HAPPENED WHEN. I KEPT FAMILY MEMBERS, YOU KNOW, APPRISED ALWAYS, YOU KNOW, OF THE PROGRESS OF THE PLAN AND ANY CHANGES THAT OCCURRED. I MEAN, I DON'T HAVE A NOTE HERE.

Q. SO YOU FEEL LIKE YOU DID AT SOME POINT TELL ONE OF THE FAMILY MEMBERS THAT CHANDLER WAS NO LONGER WILLING TO TAKE WANDA.

A. YES, AND IT MIGHT HAVE BEEN WHEN I, YOU KNOW, WHEN I HAD FOUND A NURSING HOME THAT WOULD, SINCE IT ALL CAME SO CLOSE TOGETHER HERE.

Q. LET'S GO TO THE NEXT DAY, WHICH IS JULY THE 23RD, AND OUR "EXHIBIT K", AND ASK YOU TO REVIEW THOSE NOTES. PLEASE.

A. (READING.) "CHANDLER NURSING HOME HAS OFFICIALLY REJECTED PATIENT; MAYFAIR NURSING HOME, INDIANAPOLIS, HAS EVALUATED PATIENT AT OUR FACILITY 7-22, AND SOUTHSIDE NURSING HOME, INDIANAPOLIS, WILL EVALUATE PATIENT TODAY. SHOULD HAVE RESPONSE LATE 7-23 OR EARLY 7-24. NEW MEDICO REHAB ALSO STILL FOLLOWING BUT REQUIRES "G" OR "J" TUBE."

Q. ALL RIGHT, AND THEN THE LAST PAGE OF THAT, WHICH I BELIEVE IS JULY 24, AND "EXHIBIT L." WOULD YOU REVIEW THAT, PLEASE?

[p. 31] A. (READING.) "PATIENT TO BE TRANSFERRED TO CRESTVIEW HEALTH CARE, INDIANAPOLIS, TODAY AT 2:30 P.M. HAVE NOTIFIED

PATIENT'S AUNT, JANE ROBERTS. UNABLE TO CONTACT PATIENT'S HUSBAND. WILL SEND LETTER WITH SPECIFIC DISCHARGE INFORMATION TO MR. JOHNSON. CHART COPIED TO SEND. COMMONWEALTH ACLS AMBULANCE TO TRANSPORT PATIENT AT 2:30 P.M."

Q. OKAY, MS. FRED, CRESTVIEW HEALTH CARE FACILITY, THAT'S REFERRED TO FOR THE FIRST TIME ON JULY 24, WHAT DID YOU KNOW ABOUT THAT FACILITY PRIOR TO JULY 24 OF 1992?

A. JULY 22ND, CAN I REFER BACK HERE, BECAUSE –

Q. CERTAINLY.

A. – CRESTVIEW AND MAYFAIR ARE PART OF THE SAME CHAIN, RUN BY THE SAME MANAGEMENT. OKAY, ON 7-22, MY NOTE SAYS, IN INTERIM, HAVE ASKED MAYFAIR NURSING HOME, INDIANAPOLIS, TO EVALUATE PATIENT. THEY DID SO THIS P.M. SO THE PERSON WHO WAS THERE, WHO EVALUATED THE PATIENT, WHO REPRESENTED MAYFAIR, ALSO REPRESENTED CRESTVIEW, AND SHE, JOANNA NOLTE, GAVE US A LITTLE IN-SERVICE ON HER HOSPITAL, PROVIDED SOME LITERATURE ABOUT WHAT CAPABILITIES THEY HAD, THERAPIES, AND EVALUATED THE PATIENT AT THAT TIME.

Q. NOW, WHEN SHE PROVIDED THAT INFORMATION TO YOU, WAS THAT INFORMATION RELATIVE TO MAYFAIR OR TO CRESTVIEW, OR DO YOU RECALL?



A. ALL OF, I THINK THEY HAD LIKE SIX DIFFERENT NURSING HOMES ALL IN THE INDIANAPOLIS AREA, SO SHE TALKED ABOUT [p. 32] THEIR WHOLE PROGRAM, MAYBE INDIVIDUALIZED A LITTLE BIT, YOU KNOW, AS SOME OF THE NURSING HOMES MIGHT SPECIALIZE IN, I DON'T KNOW, CLOSED UNIT FOR ALZHEIMER'S OR SOMETHING LIKE THAT.

Q. DO YOU RECALL SPECIFICALLY WHAT SHE MAY HAVE TOLD YOU THAT CRESTVIEW OFFERED PATIENTS?

A. NO.

Q. HAD YOU, PRIOR TO THIS TIME, HAD ANY EXPERIENCE REFERRING PATIENTS TO MAYFAIR OR CRESTVIEW OR ANY ONE OF THAT NATURE?

A. NO.

Q. HOW DID YOU FIRST LEARN THAT THIS HOSPITAL OR THIS NURSING HOME GROUP WAS INTERESTED IN ACCEPTING TRANSFERS FROM THE HOSPITAL?

A. JOANNA AND SOMEONE ELSE FROM HER ORGANIZATION, I BELIEVE, CAME DOWN TO US WITH THAT INTENTION, TO COMMUNICATE THAT THEY HAD SIX NURSING HOMES, AND THEY HAD AVAILABLE BEDS, AND THEY HAD SERVICES TO PROVIDE ON THE 22ND.

Q. OKAY, SO ON JULY 22ND, THEY CAME TO THE HOSPITAL TO RECRUIT OR, I GUESS, TO DISCUSS WITH THE HOSPITAL THE AVAILABILITY OF THEIR FACILITY.

A. THAT'S RIGHT, YES.

Q. AND THAT WAS THE FIRST TIME THAT YOU HAD EVER HEARD OF THIS FACILITY? WAS THAT THE FIRST TIME?

A. I THINK SO, YOU KNOW; THAT'S NOT, I MEAN, I DON'T [p. 33] REMEMBER EXACTLY.

Q. WERE THERE OTHER PEOPLE WHO ATTENDED THIS CONFERENCE WITH YOU WITH JOANNA NOLTE WHEN SHE DISCUSSED WHAT THEIR FACILITY HAD TO OFFER?

A. RIGHT, THE OTHER SOCIAL WORKERS, THE OTHER DISCHARGE PLANNERS AND THE DIRECTOR.

Q. ALL RIGHT, SO THERE WERE SEVERAL OF YOU THAT THEY PUT ON A PRESENTATION TO APPARENTLY.

A. YES.

Q. AND THIS WAS ALL ON JULY 22ND.

A. YES.

Q. AND WAS JOANNA NOLTE SCHEDULED TO EVALUATE PATIENTS THAT DAY? HAD YOU, PRIOR TO THIS, SCHEDULED WITH HER EVALUATIONS OF VARIOUS PATIENTS AT THE HOSPITAL?

A. NO.

Q. ALL RIGHT. SO AFTER, TELL US HOW SHE CAME TO PERFORM THESE EVALUATIONS THAT DAY.

A. SHE ASKED US, AS A GROUP, YOU KNOW, AFTER HER PRESENTATION, IF WE HAD, IF THERE



WERE ANY PATIENTS THAT SHE COULD EVALUATE WHILE SHE WAS HERE ON SITE, SO –

Q. AND DO YOU REMEMBER –

A. – SEVERAL OF US, I'M SURE.

Q. OKAY. DO YOU REMEMBER HOW MANY PATIENTS THAT SHE EVALUATED THAT DAY?

A. NO.

[p. 34] Q. DID SHE EVALUATE MORE OR JUST ONE?

A. JUST ONE, YES.

Q. SHE EVALUATED ONLY ONE OF YOUR PATIENTS THAT DAY, I TAKE IT.

A. YES, JUST ONE.

Q. AND THAT PATIENT WAS WANDA JOHNSON.

A. YES.

Q. NOW, DO YOU HAVE ANY INFORMATION AS TO WHETHER THE HOSPITAL OR ANY OF THE OTHER SOCIAL WORKERS HAD EVER REFERRED PATIENTS TO MAYFAIR OR CRESTVIEW NURSING HOMES PRIOR TO THIS?

A. I REALLY DON'T KNOW. NONE OF US HAD WORKED WITH JOANNA BEFORE. THAT DOESN'T MEAN ONE OF US HADN'T WORKED WITH ONE OF THOSE NURSING HOMES. IN FACT, IT'S LIKELY THAT SOMEONE HAD, BUT NO ONE HAD WORKED WITH THIS MANAGEMENT TEAM. WE, YOU KNOW, WE DON'T ALWAYS KNOW WHAT NURSING HOMES THE

OTHER WORKERS ARE WORKING WITH, NOT ALWAYS.

Q. NOW, WHEN JOANNA NOLTE CAME TO THE HOSPITAL ON JULY 22ND AND GAVE YOUR STAFF THIS PRESENTATION, WAS THIS SOMETHING THAT HAD BEEN SET UP PRIOR TO THE 22ND? HAD SHE CALLED AND MADE AN APPOINTMENT TO DO THIS, OR DO YOU KNOW?

A. I FEEL, I DON'T KNOW FOR SURE, BUT I FEEL THAT'S THE WAY IT'S USUALLY HAPPENED.

Q. AND WHO WOULD SHE HAVE CALLED TO MAKE THOSE ARRANGEMENTS?

* * *

[p. 36] OTHER NURSING HOMES IN THAT CHAIN WILL STILL ACCEPT TRANSFERS FROM YOUR HOSPITAL?

A. WELL, THEY DID. THEY DID THROUGHOUT THE NEXT 12 MONTHS, I KNOW. I WOULD ASSUME THEY WOULD.

Q. MS. FRED, YOU INDICATED EARLIER IN THE DEPOSITION THAT PART OF YOUR RESPONSIBILITY AS A SOCIAL WORKER WAS TO MAKE PLACEMENT REFERRALS FOR PATIENTS WHO COULDN'T RETURN HOME, AND THAT PART OF THAT WAS TO INQUIRE INTO AND SEE THAT THE REFERRAL SITES THAT YOU MAKE CAN PROVIDE THE MEDICAL CARE OR TRAINING OR MEDICAL CARE OR REHABILITATION SERVICES THAT THESE PATIENTS REQUIRE. WHAT INQUIRY DID YOU MAKE WITH MRS. NOLTE ABOUT



WHAT CRESTVIEW COULD PROVIDE FOR WANDA JOHNSON? WHAT INQUIRY WAS MADE THERE?

A. SHE, YOU KNOW, SHE COMMUNICATED INFORMATION ABOUT THE PROGRAM THAT INDICATED IT WAS A SKILLED NURSING FACILITY, CERTIFIED BY THE STATE CERTIFICATION AGENCY; THAT THEY HAD THE THERAPIES THAT MRS. JOHNSON COULD BENEFIT FROM TO WORK TOWARD HER REHABILITATION.

Q. DO YOU RECALL SPECIFICALLY WHAT KIND OF REHABILITATION OR WHAT KIND OF THERAPIES MRS. JOHNSON WAS GOING TO BE DISCHARGED IN NEED OF AT THAT TIME?

A. YEAH, AS A HEAD INJURY, COGNITIVELY IMPAIRED, SHE WOULD NEED SPEECH THERAPY, PHYSICAL THERAPY, OF COURSE, OCCUPATIONAL THERAPY.

BY MR. THOMPSON: SHE WAS GIVING YOU A LIST, [p. 37] WHEN YOU ASKED HER, WHEN YOU ASKED THE FOLLOW-UP QUESTION, AND –

Q. OKAY.

BY MR. THOMPSON: – I DON'T MIND IF YOU GO ON, BUT I JUST WANT TO –

Q. OKAY. YOU'VE GIVEN ME A LIST OF THREE TYPES OF THERAPIES THAT MRS. JOHNSON NEEDED AT THE POINT OF HER DISCHARGE.

BY MR. THOMPSON: I'M SORRY, SHE HAD GIVEN YOU A LIST OF WHAT INQUIRIES SHE HAD MADE.

Q. OKAY.

BY MR. THOMPSON: I THINK SHE WAS, THE LAST ONE SHE GAVE YOU WAS, IN THE HEAD THERAPY, SHE WAS IN NEED OF, I THINK SHE WAS GETTING READY TO CONTINUE WHEN YOU ASKED HER THAT.

Q. OKAY. ALL RIGHT, LET ME MAKE SURE I UNDERSTAND YOU. THIS LIST OF THERAPIES, SPEECH, PHYSICAL AND OCCUPATIONAL THERAPIES, DID YOU INQUIRE OF JOANNA NOLTE SPECIFICALLY AS TO WHETHER OR NOT HER NURSING HOMES COULD PROVIDE THESE SERVICES TO WANDA JOHNSON?

A. YES.

Q. YOU DID.

A. YES.

Q. AND DID SHE ADVISE YOU THAT THEY COULD DO THAT?

A. YES.

[p. 38] Q. DO YOU –

A. ALSO, I'M SORRY –

Q. GO AHEAD.

A. – SPIRATORY.

Q. NOW, DID SHE SAY THAT THEY COULD DO THESE AT MAYFAIR OR CRESTVIEW, OR DID SHE SAY SPECIFICALLY WHICH NURSING HOMES SHE WAS SPEAKING OF?



A. WHEN WE ORIGINALLY TALKED, WHEN SHE EVALUATED MRS. JOHNSON IN THE UNIT AND LOOKED AT THE CHART, SHE WAS WORKING TOWARD MAYFAIR. WHEN SHE COMMUNICATED THE DECISION THAT THE TEAM HAD DECIDED THAT WANDA'S NEEDS COULD BEST BE MET AT CRESTVIEW, SHE INDICATED THAT THEY HAD THE NECESSARY MODALITIES AS WELL.

Q. DID JOANNA NOLTE EVER TELL YOU THAT WANDA JOHNSON'S NEEDS EXCEEDED HER AUTHORITY TO AUTHORIZE TRANSFER OR ACCEPTANCE AT HER NURSING HOME?

A. NO.

Q. SHE DIDN'T TELL YOU THAT.

A. OH, NO.

Q. IS THERE ANYONE ELSE THAT YOU SPOKE WITH AT CRESTVIEW OR MAYFAIR OR THE PARENT COMPANY ABOUT WANDA JOHNSON'S REFERRAL TO THEIR FACILITY?

A. IN THE COURSE OF THE NEXT TWO DAYS, BETWEEN THE EVALUATION THAT SHE MADE AND THE TIME THAT WANDA WAS DISCHARGED, I HAD SPOKEN SEVERAL TIMES WITH RANDY GROSS, WHO

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[p. 41] A. YES.

Q. DO YOU REMEMBER, WAS THAT PHONE CALL THE DAY OF, DO YOU REMEMBER WHEN THAT PHONE CALL TOOK PLACE?

A. I DON'T. I'M SURE I DIDN'T TALK TO HIM ON THE 22ND. I'M SURE I TALKED TO HIM ON THE 23RD AND PROBABLY ON THE 24TH AS WELL.

Q. DO YOU REMEMBER WHAT THE SUBSTANCE OF THOSE CONVERSATIONS WAS?

A. NO.

Q. WHEN DID YOU FIRST ADVISE ANY OF THE FAMILY MEMBERS, ANY OF WANDA'S FAMILY MEMBERS, OF HER TRANSFER TO CRESTVIEW?

A. WELL, SINCE I DIDN'T REALLY KNOW FOR SURE UNTIL THE 23RD, IT COULDN'T HAVE BEEN BEFORE THEN, SO IT WAS THE 23RD AND/OR THE 24TH.

Q. SO YOU SAY YOU TALKED TO SOMEBODY ON THE 23RD AND ON THE 24TH.

A. AND/OR.

Q. ALL RIGHT. DO YOU KNOW WHO YOU TALKED TO?

A. YEAH, IT WAS THE AUNT, JANE ROBERTS.

Q. AND YOU SPOKE WITH HER PRIOR TO HER BEING TRANSFERRED?

A. YES.

Q. DID YOU EVER ATTEMPT TO GET A-HOLD OF WANDA'S MOTHER OR HER HUSBAND?

[p. 42] A. I HAD, YEAH; I TRIED DAILY OR AT ANY TIME A DECISION POINT WAS REACHED TO GET IN TOUCH WITH MR. JOHNSON. YOU KNOW,



THIS WAS REAL IMPORTANT. YOU KNOW, TO TRY TO REACH THE NEXT OF KIN. I WAS CONCERNED ABOUT IT, AND I DID TRY, BUT THE AUNT HAD BEEN THE MOST HELPFUL OF THE FAMILY MEMBERS' THAT OFFERED THE MOST INFORMATION. WANDA'S MOTHER DIDN'T SEEM TO WANT TO BE INVOLVED IN ANY DECISIONS.

Q. DO YOU FEEL LIKE WANDA'S AUNT HAD AUTHORITY TO AUTHORIZE A TRANSFER TO INDIANAPOLIS?

A. SHE WAS THE FAMILY MEMBER THAT WE WERE IN COMMUNICATION WITH. AND EVERYBODY ELSE HAD ABSCONDED OR ELIMINATED THEMSELVES.

Q. WHEN YOU SPOKE WITH THE AUNT, DO YOU REMEMBER WHAT SHE SAID OR ANY COMMENTS THAT SHE MAY HAVE MADE CONCERNING THE TRANSFER WERE WHEN YOU ADVISED HER OF THIS MAYFAIR/CRESTVIEW DECISION?

A. WE HAD BEEN IN COMMUNICATION CONTINUOUS FROM THE TIME, YOU KNOW, THAT I FIRST REACHED HER. I HAD, YOU KNOW, IT'S AN EDUCATED PROCESS WITH FAMILY MEMBERS, WHEN YOU'RE INVOLVED IN THE PLACEMENT OF A PATIENT, AND YOU CAN'T PUT THEM IN THEIR BACK YARD. IT'S IMPOSSIBLE AND SO, YOU KNOW, YOU'RE CONSTANTLY TRYING TO EDUCATE THEM TO WHAT THE PROBLEM IS AND THE LIMITATIONS OF THE SYSTEMS, AND YOU KNOW, WHY CHANDLER, EVANSVILLE AND, YOU KNOW. I'M SURE SHE EXPRESSED REGRET AT THE DISTANCE,

AND I EXPRESSED REGRET ALSO, AND SHE [p. 43] MAY HAVE, SHE PROBABLY SAID, WHAT ABOUT FEDERAL HILL OR, YOU KNOW, AND I'M SURE I SAID, I'LL CALL THEM. I KNOW THEY DON'T HAVE ANY BEDS, BECAUSE WE'VE SURVEYED REGULARLY, AND THESE THINGS ARE THINGS I DON'T SPECIFICALLY REMEMBER, BUT I'M SURE I WOULD HAVE ENCOURAGED HER TO PUT HER ON ANY LISTS, YOU KNOW, CLOSE AT HAND.

Q. DID SHE HAVE AN OPTION AT THAT POINT? HAD MRS. ROBERTS, HER AUNT, TOLD YOU, YOU KNOW, WE'RE NOT GOING TO GO ALL THE WAY TO INDIANAPOLIS; FIND SOME PLACE ELSE.

A. SHE DID, AND SHE DID NOT SAY THAT.

Q. HAD SHE SAID THAT, WHAT WAS THE HOSPITAL'S POSITION?

A. WELL –

BY MR. THOMPSON: LET ME OBJECT TO THE QUESTION. I THINK IT'S KIND OF SPECULATIVE. BUT GO AHEAD AND ANSWER IT.

A. WELL, AS THE ONLY FAMILY MEMBER LEFT THAT WE COULD CONTACT, THE PERSON THAT WE WERE WORKING WITH, IF SHE HAD SAID, I OPPOSE THIS, WE WOULDN'T HAVE DISCHARGED HER TO THAT FACILITY.

Q. JUST A COUPLE OF MORE QUESTIONS, AND WE'LL TAKE A BREAK. YOU HAD BEEN WORKING WITH NEW MEDICO –

A. YES.



Q. – THROUGHOUT THIS PROCESS, TRYING TO GET SOMETHING WORKED OUT WITH THEM. COULD YOU TELL US WHY YOU DIDN'T WAIT

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[p. 54] Q. SO YOU'RE SAYING THAT –

A. I MEAN. THAT'S, YOU KNOW, I'M NOT A DOCTOR.

Q. ALL RIGHT, SO YOU'RE SAYING THAT FOR WANDA TO HAVE STAYED IN AN ACUTE SITUATION WOULD HAVE CONSTITUTED SOME TYPE OF RISK TO HER?

A. WELL, THE DOCTORS DETERMINE WHEN A PATIENT IS READY FOR DISCHARGE, YOU KNOW, AND THEY SAID SHE WAS.

Q. WHO DID YOU CONSULT WITH? YOU INDICATED THAT THE DECISION WAS NOT ALWAYS YOURS AS TO WHERE A PLACEMENT IS MADE, AS BETWEEN REHAB AND A NURSING HOME. BETWEEN THE 22ND OF JULY WHEN YOU MET WITH JOANNA NOLTE, AND THE 24TH WHEN WANDA WAS TRANSFERRED, DO YOU REMEMBER, DID YOU CONSULT WITH ANYONE ELSE ON THE STAFF ABOUT WHAT WERE WANDA'S OPTIONS AND WHERE WAS THE MOST APPROPRIATE TRANSFER?

A. PROBABLY, I'M SURE I DID. I'M SURE I DISCUSSED IT WITH MY DIRECTOR.

Q. WHO WOULD THAT HAVE BEEN?

A. GOLDIE MURPHY.

Q. ANYONE ELSE THAT YOU THINK YOU MAY HAVE CONSULTED WITH DURING THAT TIME?

A. JUST SHARING WITH COLLEAGUES.

Q. WOULD THERE HAVE BEEN ANY CONSULTATION AT ALL WITH ANY OF THE MEDICAL STAFF ABOUT, HERE'S WHAT WE'VE GOT TO OFFER THIS PATIENT; WE'VE GOT CRESTVIEW ON THE ONE HAND, AND WE'VE GOT NEW MEDICO, WHO STILL HASN'T AUTHORIZED THE TRANS- [p. 55] FER? ANY CONSULTATION AT ALL WITH ANYBODY ON THE MEDICAL STAFF DURING THAT PERIOD OF TIME?

A. I'M SURE I WAS SPEAKING WITH THE DOCTORS.

Q. AND WHICH DOCTORS WOULD THAT HAVE BEEN?

A. OH, PROBABLY DR. ABOU JAOUDE.

Q. IS HE STILL WITH THE HOSPITAL?

A. I BELIEVE HE IS.

Q. AND YOU BELIEVE YOU DISCUSSED WITH HIM –

A. YES.

Q. – YOUR TRANSFER STATUS?

A. HE AGREED WITH IT, YOU KNOW, FROM THE INFORMATION THAT HE HAD AND FROM, I DON'T KNOW IF DR. A. J. TALKED WITH JOANNA WHEN SHE WAS THERE. I DON'T REMEMBER.

Q. BUT YOU THINK THAT YOU DID TALK TO THE DOCTOR BETWEEN THE 22ND AND THE 24TH



AND ADVISED HIM OF THIS PROSPECTIVE TRANSFER.

A. YES.

Q. WHAT DID YOU KNOW ABOUT WANDA'S PAY STATUS, AS OF JULY 24?

A. I KNEW THAT SHE WAS WITHOUT INSURANCE, INDIGENT, MEDICALLY INDIGENT, AND THAT HER HUSBAND HAD APPLIED FOR SOCIAL SECURITY DISABILITY, WHICH ALSO INCLUDED A KENTUCKY MEDICAID APPLICATION.

Q. AT THAT POINT, YOU STILL DID NOT KNOW WHETHER OR NOT SHE'D BEEN APPROVED FROM KENTUCKY MEDICAID?

[p. 56] A. DID NOT.

Q. DO YOU HAVE, THE SOCIAL WORKER STAFF HAVE, I GUESS, DISCHARGE STANDARDS, SOMETHING THAT YOU LOOK AT TO SAY, HERE'S WHAT YOU LOOK TO TO DECIDE WHETHER A PATIENT IS APPROPRIATE FOR TRANSFER, OR THIS IS THE KIND OF TRANSFER, THE KIND OF REFERRAL THAT YOU MAKE ON A PARTICULAR KIND OF PATIENT? ARE THERE ANY KIND OF STANDARDS LIKE THAT THAT YOU HAVE THAT YOU GO BY?

A. I'M NOT SURE WHAT YOU MEAN. THERE ARE POLICIES AND PROCEDURES.

Q. EXACTLY, ARE THERE POLICIES AND PROCEDURES THAT YOU USE TO DETERMINE WHAT KIND OF PLACEMENT THAT YOU MAKE ON DISCHARGED PATIENTS?

A. WELL, I DON'T UNDERSTAND YOUR QUESTION. I'M SORRY.

Q. DO YOU HAVE A POLICY MANUAL AT THE HOSPITAL THAT EXPLAINS THE FACTORS THAT YOU CONSIDER WHEN DECIDING WHICH OF VARIOUS ALTERNATIVES, IN DISCHARGE PLACEMENTS, THAT YOU SHOULD MAKE ON A GIVEN PATIENT?

A. YOU KNOW, IT'S BEEN SO LONG SINCE I'VE LOOKED AT THEM, I'M NOT SURE. I'M SURE, YES.

Q. THAT YOU THINK THOSE POLICIES EXIST.

A. AND MANY OF THE POLICIES ARE COMMON SENSE, OR THEY'RE STATE-REGULATED. FOR EXAMPLE, YOU CAN'T SEND A PATIENT WHO NEEDS INTERMEDIATE CARE TO A PERSONAL CARE PLACEMENT. THAT'S A VIOLATION OF THE STATE CERTIFICATIONS.

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UNITED STATES DISTRICT COURT
WESTERN DISTRICT OF KENTUCKY
LOUISVILLE DIVISION
CAUSE NO. C-93-0544L-M

[Caption Omitted In Printing]

The video deposition upon oral examination of JO ANNA NOLTE, * * * on the 24th day of January, 1994, * * *

* * *

[p. 22] June or July of 1992 in your visit to the University Hospitals, University of Louisville staff, tell us what happened, what – what did you do on that first visit?

A I met with them as a group and there were quite a few social workers there, maybe eight to ten of the social workers were there. I spoke to them about our different facilities; of course, introduced myself. I took them lit – any literature at that time.

Q What type of literature did you usually have?

A Brochure and – and my business cards and that was all we had.

Q One brochure?

A I took I believe several down so that they'd have some.

Q But there – there was one – one brochure –

A For the entire –

Q – that you brought several copies of?

A Right, and there was one brochure for the company that was used for all – all the facilities.

Q How many different types of brochures did you bring to – to Louisville?

[p. 23] A We had only one type of brochure to bring.

Q So your initial contact was when you came to Louisville and you had a meeting with several of the social workers?

A Uh-huh.

Q At that time – let me – let me back up. Do you remember whether that was in June or July or can you give us an approximate date?

A I – I really can't. I don't – I don't remember if – if the time that we looked at Wanda was my first time or my second time down there, I really – I really don't remember, I'm sorry.

Q Okay. Tell us how you first learned of a patient named Wanda Johnson.

A The social worker that was handling her case asked me if I would be willing to take a look at a patient that they had been trying to discharge and without any avail to other facilities and that she was getting a lot of pressure to discharge this person due to they knew they weren't going to get paid for this person.

Q Do you recognize the name Nancy Fred?

A Correct, it was Nancy Fred.

[p. 24] Q And Nancy Fred is the person who you had this conversation with?

A Correct.



Q And was it Nancy Fred who initially asked you to look at Wanda Johnson's situation?

A Yes.

Q Do you remember whether that conversation with Nancy Fred took place at the first visit down to Humana or the second?

A I don't remember. It was – I – I really don't. I know it was the day that I took a look at Wanda and gathered information and took back to the facility.

Q All right. The day that Nancy Fred had spoken to you initially about Wanda Johnson, had you evaluated or taken a look at other patients that day?

A I think they had me look at maybe one other. I – it's – you know, when you're a marketing person, you're making so many marketing calls it's hard to remember how many you look at at each visit.

Q When you would look at a – at a prospective resident in a hospital, what kinds of things were [p. 25] you looking for?

A Well, the nurses would want me to – and each DON was different as far as what kind of information that they would want me to get copies of them for them. They would want information about medication, level of care, that a resident's cognitive ability, whether they're ambulatory, size of resident, those type of things.

Q Do you recall whether or not you had been scheduled to see this other patient, the one other than Wanda, had you been scheduled to see her prior to the

time you came down or was that also someone that they asked you to see once you got there?

A I think it was kind of spur of the moment.

Q All right. Do you recall what, if anything, Nancy Fred told you about Wanda Johnson's physical condition and her physical needs during that initial visit?

A That she was very heavy care, that she was starting to respond some, that they were hoping that she would eventually be able to – to be rehabilitated, that she had a lot of problems [p. 26] with temperature, maintaining temperature and I think – well, respiratory problems in the past were some of the things that they were struggling with, some family issues, things like that.

Q What did Mrs. Fred tell you about Wanda's family status?

A She said that the husband had – had left and I believe moved in with a girlfriend possibly, that Wanda's family then, mother or aunt, was then taking care and taking the responsibility for her, that Mr. Jackson really wasn't coming around too much.

Q Now, you said Mr. Jackson?

A Or her husband, Wanda's.

Q Mr. Johnson?

A Mr. Johnson, sorry.

Q Did Nancy Fred indicate to you anyone in – in particular who was – who was placing pressure on her to discharge Wanda?



A She didn't mention any names, but she said she was just receiving pressure from the hospital that, you know, that we needed – that she needed to find placement for this person.

[p. 27] Q Did she tell you or – or name any of the other facilities that had considered Wanda and turned her down?

A She named several but I can't remember them. You know, I think she named a couple that – that she had worked with and came close to maybe admitting and then they fell through for her at the last minute.

Q Did she indicate to you the reason why those other facilities turned her down?

A She may have but I don't remember exactly what they were.

Q Tell me specifically what – as best you can recall what Mrs. Fred told you about the reasons that she needed to have Wanda Johnson transferred or discharged.

A It was basically that they had had her for several months and they – they knew that they were not going to get paid and that they needed to get her discharged.

Q Did she indicate to you whether or not application had been made for Medicaid benefits?

A I think they had attempted possibly and maybe not [p. 28] everything got done because I know the administrator at Crestview had to do a great deal of work trying to get Medicaid application made for her. So it may have been started and kind of the ball dropped or may

have been started and Randy picked up where they left off, I'm not sure what.

Q After this discussion with Nancy Fred, did you then actually have an opportunity to go in and – and look at Wanda Johnson and evaluate as best you could her condition?

A Well, I didn't – I didn't evaluate but I always like to always go in and introduce myself to people. So I did go in and I – I believe I probably told her hello, who I was, what I was there for.

Q As best you can recall, can you tell us what your observations of Mrs. Johnson's condition were on that date?

A Wanda, if I – she was a rather large woman, not real responsive but did – I remember I think when I called her name she did look at me. She really could not talk to me or answer questions [p. 29] or anything like that, but she did seem to be somewhat alert to her surroundings. Just from looking at her, not being a medical person, it was – I could tell, though, obviously that she took a lot – took a lot of care, you know, because the way they had her positioned with a lot of pillows and this and she didn't move at all while I was in there.

Q Following your evaluation, to what extent you evaluated Wanda –

A Uh-huh.

Q – do you remember what your comments to Nancy Fred or anyone else at Humana were concerning Wanda and – and your feelings concerning her transfer or her discharge?



A Well, I told Nancy that I would contact one of – one of our facilities to see if they would be able to take her, you know, that I could not make any promises while I was there because it would be up to the facility as to whether or not she could be accepted into them and that we would, you know, get back with her as soon as we could.

Q Were you required at that point to take [p. 30] information from the hospital and deliver it to someone at Hometown?

A Correct. I did – I called the – the Mayfair facility which was Hometown's largest facility and the DON gave me a list of – of documentation that she would like me to have Xeroxed from the hospital to bring back with me to help her with evaluating how many nursing hours and the level of care that would be required for Wanda.

Q Now, was this call made from Humana Hospital in Louisville?

A Uh-huh, I think so. I believe I did.

Q And was it made shortly after your actual evaluation of Wanda?

A Yes.

Q Who was the director of nursing at Mayfair at that time, do you remember?

A It's on the tip of my tongue. No, I can't remember what her name was. If I heard it, I'd say yes, that was – I'm really bad with names, I'm sorry.

Q All right. So in any event, you called whoever the director of nursing was at Mayfair?

[p. 31] A Uh-huh.

Q And they gave you a list of documents that they needed?

A Correct, that they would like to have.

Q And did you get those documents from someone at Humana?

A I gave Nancy the list of items that we wanted and she took care of putting together a packet for me to take back with me.

Q Did she put that packet together for you to take back on that date?

A I believe, yes.

Q Did Nancy Fred or anyone else at Humana tell you anything else about Wanda Johnson during your visit there in Louisville or express to you any other opinions or – or encouragement with respect to accepting Wanda, anything of that nature?

A Well, they said, you know, we hope you can really help us out and, you know, typical when a – typical kind of response you get from a discharge planner when they're struggling and panicked to find placement.

[p. 32] Q And would these statements also have been made by Nancy Fred?

A Yes.

Q Was she the only person that you talked to at Humana concerning Wanda Johnson?

A I believe so.



Q So if I understand you correctly, you left Humana Hospital with a package of information concerning Wanda Johnson?

A Correct.

Q Who did you deliver that information to?

A It was delivered to the DON at Mayfair.

Q Do you know how long after your visit to Louisville that was delivered?

A I – I drove right back to Indianapolis that day so if I got back before quitting time it went that day.

Q Otherwise?

A It would have gone the very next morning.

Q After you delivered the information to the director of nursing at Mayfair, did – did that director of nursing follow up with any phone calls to you inquiring about anything else [p. 33] concerning Wanda Johnson's condition?

A I don't believe so. She may have called down there and talked to the nursing on the – on the unit.

Q But as far as the director of nursing talking to you, she did not?

A I don't believe so.

Q Following your trip back up to Indianapolis from Louisville when you received that information about Wanda, do you recall when it was that someone within your facility next talked to you concerning Wanda Johnson?

A Could you say that again.

Q I'm sorry.

A I – I didn't get the question.

Q After you had delivered the information from Humana to the director of nursing at Mayfair –

A Uh-huh.

Q – were you involved any longer in the process of determining whether or not any of your facilities would accept Wanda Johnson?

A No, you know, they might ask me another question or a name or a phone number, but once I give them [p. 34] the referral, I'm done with the case and it's up to the facility to make the determination whether or not they want to admit or not. Mayfair chose not to because they were – they had a great deal of number of skilled individuals in the facility and did not want to take another skilled patient at that time. And then it was determined I believe by, again, Ms. Tobey that we would pass the referral on to Crestview.

Q Now, were people from Mayfair or from Hometown contacting you in the interim and updating you on where we were with respect to Wanda Johnson's referral?

A Yes, as common courtesy, being the marketing director, it was also my job to find out, you know where are we with referrals, where are we with admissions, what's the bed count, things like that. So they would keep me updated.

Q Did you relay to anybody at Mayfair or Hometown Living Centers the comment made by Nancy Fred



that they were not getting – getting paid for Wanda Johnson, did you tell anybody within your facility or your organization about that problem

* * *

[p. 45] might want to start looking around to make sure I had something stable.

Q When you were with the company, were you on a salary or were you paid a commission based on referrals?

A A salary plus commission.

Q And how did the commission work?

A Commission would work that I would find their referral, if the admission panned out and the resident maintained a number of days in the facility and the company received payment, then I would receive a bonus.

Q And tell me how the bonus worked or was it based on a percentage or –

A It was just a flat, a dollar amount per resident.

Q And how much was it?

A Three hundred.

Q Three hundred?

A And then Daphna changed policy at the beginning of '93 that bonuses would be split between me and the administrator.

Q Prior to that time, the administrator did not get a bonus?

* * *

[p. 51] remember regarding your discussions with Nancy Fred at the University of Louisville Hospital?

A Yeah, I believe so.

Q Okay. I take it you didn't tell Nancy Fred that Crestview had never received a patient, what you called heavy care patient, this was its first; correct?

A It was their first extremely heavy care patient.

Q And you didn't tell that to Nancy Fred or Nancy Fred to your knowledge wasn't aware of that?

A I told Nancy that we did have skilled patients in the building and we have had skilled patients in the past.

Q The initial conversation with Nancy Fred related to Mayfair, though, did it – didn't it?

A Correct.

Q Okay. And you told us that after the patient was referred to Mayfair and you came back here you didn't have any further contact with Nancy Fred; is that correct?

A When Mayfair would not take her, I had to call Nancy and tell her what facility I was referring it to.

[p. 52] Q Okay.

A That's common courtesy.

Q Okay. And – and at that time, you didn't tell Nancy Fred that Crestview had never taken a – or this was its first heavy care patient, you – you simply told her it was a skilled nursing care facility?



A Uh-huh, and they would have to evaluate her whether or not they would be able to take her.

Q All right. The conversation you had with – is it Ms. Daph – Daphie, is that the name?

A Daphna.

Q Daphna, I'm sorry.

A That's okay.

Q I'm been mispronouncing it. Daphna, did Ms. – Ms. Daphna give you some reason why she thought Ms. Ferguson may be reluctant to receive this patient?

A No.

Q Okay. Was it customary for Ms. Daphna to tell you to tell the director of nursings at – at these facilities to have an open mind on – with respect to patients?

* * *

[p. 64] Q Okay. Crestview in – in July of '92 had the capability of administering IV medications; correct?

A That's my understanding that they did have IV certified nurses.

Q Okay. We've talked a little bit about heavy care and I'm going to represent to you that Ms. Johnson when she was transferred to Crestview had a tracheostomy, she had a foley catheter and a nasogastric feeding tube, would that meet what you are using the term heavy care?

A Correct.

Q And you told us that Mayfair had those patients before July of '92 and this was Crestview's first such patient?

A Well, they had had patients with tube feedings in the past, they had had patients with catheters in the past as well, but they did not have I don't think anyone with all the multiple things at that time. They had patients there at the time that had some of the other items that would classify them as skilled care.

Q Do you recall when you talked to Ms. Ferguson,

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UNITED STATES DISTRICT COURT
WESTERN DISTRICT OF KENTUCKY
LOUISVILLE DIVISION
CAUSE NO. C-93-0544L-M

[Caption Omitted In Printing]

The video deposition upon oral examination of CHARNA Y. TRESER, • • • on the 25th day of January, 1994, • • •

• • •

[p. 63] about that six p.m. entry on your nursing notes. That refers to the orders that you received from Dr. Shatara. The first order that you received was apparently to determine Mrs. Johnson's code status; is that correct?

A Uh-huh.

Q Now, explain to the jury just in case they don't understand, what do you mean by full code, the various levels of code that – that we're discussing?

A Okay. A full code is that the – the family would request that a resident if they go in distress or – or have a problem where they need to be seen in a better equipped facility do all that you can for them.

Q And that's full code?

A Yes.

Q All right.

A A no code is the family would like for that person if they're in a distress or a situation where they may be RHC –

Q And what's RHC?

A They may de – be deceased.

[p. 64] Q Okay.

A Or will get to that point where they're – they want them to go on their own with no help at all, just make them comfortable.

Q All right. So in a no code situation, you make the patient comfortable?

A And –

Q And wait?

A – you would monitor them and just let them go at their own.

Q And let them expire if that's what happens?

A Yeah, on their own.

Q Now, was it unusual for you in receiving a patient in at Crestview and when you made your initial call to the doctor on – on call, was it unusual that the first order that he would give to you was to determine the code status for that patient?

A (No response).

Q Let me – let me rephrase the question maybe. Do you – do you recall ever –

A I understand the question.

Q Okay.

[p. 65] A I'm just trying to see how I would phrase it.



Q Okay. Well, let – take your time.

A I've never had that said to me before.

Q Okay. So it was unusual that that was his first order?

A Well, to me, I never had it said. It might have been said to RNs, you know, because every – every resident has to have a code status, you know, I mean that's the patient family's right. That was the first time it was said to me.

Q So normally when you make that initial contact to the doctor when you're admitting a patient other orders are given other than the request for determination of code status?

A Yes.

Q Do you know why in Mrs. Johnson's case that was Dr. Shatara's first order to you?

A No, I don't.

Q Based on your evaluation of Wanda, do you think or do you have an opinion as to whether or not it had anything to do with her condition when she was presented to the nursing home?

A My opinion?

[p. 66] Q Yes.

A Yes, he probably wanted to know the code because, you know, the condition she was in.

Q Now, apparently after you talked to Dr. Shatara there was a call that was made to one of Wanda's family

members and eventually I believe at seven-forty-five, although it's not in – in the notes that you took, there is some indication that a family member had called and discussed with someone her code status. Do you remember someone relaying to you what Wanda's family had told the people at Crestview concerning code status for Wanda?

A Do I remember?

Q Do you remember what her code status was after –

A After she talked to –

Q Correct.

A I can't remember if it was told to me or not because at this time Lynnette took over to a certain extent, you know, what was going on.

Q Ms. Treser, was Wanda Johnson's condition when she presented herself or was presented to your nursing home, was it more severe than any other [p. 67] patient that you can ever remember accepting at Crestview Nursing Home?

A Her condition, yes, it was. She was in – to my – of my opinion, she was in pretty bad shape.

Q And isn't it true, Mrs. Treser, that her condition was so severe when she was presented to your nursing home that you thought she was going to die before she was able to be placed someplace else?

A Let's say that could have been a possibility.

Q You thought of that, did you not?



A Yes.

Q And wasn't that the reason that it was so important to determine her code status?

A Yes.

MR. MATTINGLY: Thank you, Ms. Treser.

REDIRECT EXAMINATION,

QUESTIONS BY B. TODD THOMPSON:

Q Ms. Treser, I have just a few more questions. As I understand it, all patients at Crestview had a code status?

A Yes. Everyone has a code status, yes.

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[p. 69] A I have some that were admitted two-thirty, three o'clock.

Q Okay. The norm, the routine would be that most patients come in sometime before the second shift?

A I don't know if you would say routinely. Let's just say it happens that way sometimes.

Q You were asked about this patient's – Mr. Mattingly asked you some questions about Ms. Johnson's condition and whether you thought she might die and questions to that effect. We know that by – at least by eleven o'clock p.m. when you left the nursing home you charted that there were no signs or symptoms of distress for Ms. Johnson; correct?

A Yes, I did chart that.

Q Okay. And you've told us that you communicated to Dr. Shatara Ms. Johnson's condition and you deferred to his judgment as the physician as to what medical treatment or what would be done for Ms. Johnson; correct?

A Uh-huh.

MR. THOMPSON: Okay. That's all. [p. 70] Thank you.

RECROSS-EXAMINATION,

QUESTIONS BY JOSEPH H. MATTINGLY, III:

Q Ms. Treser, just one further question. In response to Mr. Thompson's question, you – you related to us the routine at Crestview Nursing Home for determining a patient's code status. Let me ask you, had – had you prior to Wanda Johnson ever admitted a patient, done the initial evaluation and actually were – were the first person at Crestview to – to admit a particular patient, had you done that before?

A No.

Q You had never done that before?

A No.

Q All right. Had you ever been prior to Wanda Johnson the first person at Crestview to talk to the doctor to get the doctor's orders?

A On a new admit, I've took and verified, yes – well, I have never did a total admit before, no, because everybody does it – the RN, if there's an RN there will do



it, you know, and if I – if she wants me to follow her up or help, I will

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UNITED STATES DISTRICT COURT
WESTERN DISTRICT OF KENTUCKY
LOUISVILLE DIVISION
CAUSE NO. C-93-0544L-M

[Caption Omitted In Printing]

* * *

The video deposition upon oral examination of JOHN W. STUY, M.D., *** on the 25th day of January, 1994, ***

* * *

[p. 7] A Several months. Several months.

Q Do you know approximately what month it was that you last saw Wanda Johnson?

A I believe it was in January of the following year that I had her discharged.

Q And was that discharge from Midwest Medical Center?

A That's right.

Q So, Doctor, was Wanda Johnson hospitalized at Midwest Medical Center during the entire time that she was your patient from July 25, 1992 through January of 1993?

A That's correct.

Q In addition, Doctor, to having treated Wanda Johnson for a period of approximately six months, have



you also had an opportunity to review some of the medical records from Crestview Nursing Home here in Indianapolis and also some of the records from the University of Louisville Hospital in Louisville, Kentucky?

A Yes, I have.

Q Could you tell us generally, Doctor, with respect, first of all, to Wanda Johnson's [p. 8] condition prior to her transfer from Louisville, Kentucky, could you tell us what you recall as being significant, significant conditions with respect to Mrs. Johnson that you were able to glean from the medical records that – that you saw from the University of Louisville?

A I saw two days of progress notes written by physicians at Humana Hospital.

Q Do you remember the dates of those admissions or those –

A 23rd and 24th of July.

A Two days prior to her transfer?

A That's correct.

Q All right.

A That's correct. I believe she was transferred on the 24th. And on both of those days, the physician writing the progress note commented on a fever that she had and that he had initiated oral antibiotic and that she also had an elevated white blood cell count. The white blood cell count when it's elevated usually reflects active infection, although there are other medical conditions that can cause it to be elevated as [p. 9] well. And on the 24th, if I

remember correctly, she had a temperature that was higher than on the 23rd.

Q Doctor, we have had some previous testimony in this case from the nurse who was on staff at the University of Louisville Hospital on the date that Mrs. Johnson was transferred and her testimony was that on the date of Wanda Johnson's transfer her – Wanda's white blood count was twenty-two thousand.

A Uh-huh.

Q Could you tell us whether a white blood count at that level is significant to you as a – as a specialist in internal medicine; and if it is, tell us why it is significant and what it – what it showed?

A Well, it's quite high. A normal white blood cell count while it varies a little bit from laboratory to laboratory what is considered their upper limit of normal, twenty-two thousand is very high, and in someone with a fever and an elevated white blood cell count, my first concern is that she has an active infection and a serious [p. 10] active infection with a white count that high.

Q All right. What, Doctor, do you generally see as – as a normal white blood count?

A Again, it can vary a little bit from laboratory to laboratory, but most upper limits of normal would be ten thousand five hundred, ten thousand eight hundred and, again, that varies from lab to lab.

Q The nurse who was present at the University of Louisville Hospital at the time that Wanda was discharged also noted that – that Wanda had cloudy urine or a hazy yellow urine, is that something that's significant to



you in reviewing records; and if so, what is the significance of that?

A Well, that could reflect active urinary tract infection, infection within the urine, in the bladder, possibly even the kidney.

Q Do you recall reading anywhere else in the medical records that you reviewed during those last two days of Wanda's stay at the University of Louisville Hospital about a urinary tract infection?

A The physician writing the progress notes commented that he felt the elevated temperature probably was a result of the urinary tract infection and that's why he initiated the antibiotic that he picked.

Q Generally, Doctor, when – when a physician is reviewing a patient's progress and notes things like we've just talked about, a white blood count that's elevated and elevated temperature, cloudy urine and when, as you said, the doctor administers or elects to administer antibiotics, is there any kind of standard in – in – of good medical practice as to the length of time that that doctor should review to determine whether that particular antibiotic is – is taking care of the problems that he's found that are evidenced by these factors that we've talked about?

MR. THOMPSON: Before he answers that, let me object to this line of questions from this treating physician who has not been identified as an expert witness to render opinions about the hospital care at University of [p. 12] Louisville Hospital. My understanding is he

is a fact witness. We haven't brought forward the outstanding requesting expert witnesses and their opinions. This has not been provided to us today, and if opinions are sought here and the court permits them, we will ask to come back and redepose the witness after we've had an opportunity to prepare to cross-examine him on expert opinions.

Q Would you like me to repeat the question, Doctor?

A Yes, I think you lost me.

Q All right. Doctor, we're – we're talking about what is expected or what is reasonable medical care in a situation where a doctor has identified what appears to be active infection and has prescribed antibiotic treatment for that infection. Is there a certain period of time that you normally would expect to have to monitor a patient to determine whether the antibiotics that you have prescribed are going to adequately treat the infection that you found?

A Well, what you would typically do is obtain a culture of the potential source of infection. So [p. 13] in this case, you would obtain a sample of urine and send it to the laboratory for culture and sensitivity; by that, the laboratory would try to grow bacteria to see if there is an active infection, identify those bacteria and then they can in the laboratory check various antibiotics and see if those antibiotics would successfully kill or eliminate those bacteria. So you identify the organism then you can actually check a number of antibiotics and see if those are successful in treating that particular infection.



Q Generally is this something that takes a period of days?

A Within 48 hours they should be able to render a preliminary report and within 72 hours they should be able to give a positive identification.

Q If a physician does not follow through with this analysis of the antibiotic treatment that he's administered, would that generally in your opinion be outside the normal standard of good medical care?

MR. THOMPSON: Same objection.

A So I can talk? I think in this particular [p. 14] patient because she is immunocompromised, she's lacking a spleen, also the fact that she has a predisposition to pneumonia, she has a predisposition to urinary tract infection because of a catheter residing in her bladder and the fact that she's been in the hospital a long time and, therefore, exposed to bacteria and organisms that are not typically found in the community, organisms that are typically more easily treated, I think she should have had a culture obtained and looked at prior to her release to be sure that this antibiotic would address that infection successfully.

(A discussion was held off the record.)

MR. MATTINGLY: To go on the record with respect to Mr. Thompson's objection to this witness expressing opinions about the care rendered by the University of Louisville Hospital, I understand that the objection is that this witness has not been identified as an expert and that that will be a continuing objection and we would have no problem if it's considered that rather than

having to interpose an objection to [p. 15] each opinion that's rendered by Dr. Stuy or any question asking for Dr. Stuy to render an opinion today.

MR. THOMPSON: With that understanding, in light of the fact that we are on a videotape, we will not interpose those objections and it's understood that they are continuing and that they are not waived.

MR. MATTINGLY: That's correct.

Q Dr. Stuy, based on your review of Wanda Johnson's medical records from the University of Louisville during the last couple of days that she was there, was her condition such that – that she required continuous medical attention in a hospital setting?

A I think, as I mentioned previously, that cultures of the presumed infection should have been obtained and looked at first prior to transfer to be sure that the antibiotics that they picked would successfully treat that infection. If they picked an antibiotic that would not successfully treat an infection and she were released, because of her generally immunocompromised state, lack of [p. 16] spleen, prolonged hospital stay, foreign bodies in the form of catheters, there is some risk that the infection would get worse and could cause her harm.

Q So, Doctor, because of that risk, did her condition require that medical personnel such as doctors and – and other people who you would expect in a hospital be able to treat her in case one of these complications arose?

A Maybe I'm not quite following your – your line of thought.



Q All right. Was Wanda Johnson's medical condition such that if she did not have available to her medical attention her condition could possibly have deteriorated very quickly?

A Yes.

Q Describe again for us the types of health risks or conditions that under Wanda Johnson's condition she may have suffered from, not necessarily that she did suffer from, but the risks that she – that were involved in her condition.

A Her ability to fight off infection was [p. 17] compromised. If the infection were not to be checked or treated with the antibiotics successfully, it could spread into the bloodstream and cause a condition called sepsis which if not treated quickly can result in death.

Q Doctor, based on your experience and treatment of Wanda Johnson and your review of her medical records at Humana Hospital, University of Louisville, do you have an opinion as to whether Wanda Johnson's transfer from Louisville to Indianapolis on July 24, 1992 was consistent with accepted medical standard of care?

A I don't think it was.

Q Doctor, would the risks that you've previously told us about, would a – a trip in an ambulance from Louisville to Indianapolis, thaf period of time that it would take to – to travel that distance, would you consider that a dangerous situation for Wanda Johnson in her condition?

A I think she should have remained in the hospital setting until they could document that the antibiotics that

they picked could successfully treat the suspected infection.

[p. 18] Q Doctor, you're familiar with Crestview Nursing Home, are you not?

A Somewhat.

Q And tell us how you are familiar with that or how you were familiar with that as it existed on July 24th and '5th of 1992?

A One of the members of my group takes care of – or took care of residents of Crestview Nursing Home and that's how I became involved in Wanda Johnson's care since I was covering for him on the weekend of the 25th. I know that they offer intermediate and skilled care. I spoke with nursing staff on that Saturday morning to get more information about her and to get some information about what they offer in terms of care. I have not physically been inside Crestview nor have I admitted residents to Crestview Nursing Home.

Q Based on your conversation with that representative from Crestview on that Saturday morning and what she told you about what Crestview was able – able to offer a resident like Wanda Johnson, did it appear to you or do – [p. 19] do you have an opinion as to whether or not Crestview Nursing Home was capable of providing the facilities and services to adequately care for Wanda Johnson in the condition that she was in when she was presented to them?

A I asked if they took care of individuals who had suffered traumatic injuries such as Wanda Johnson and the nurse that I spoke to that morning stated to me that



Wanda Johnson was the very first patient that they had ever received which suffered any sort of traumatic injuries of the nature that she had.

Q Doctor, given what you knew about Crestview Nursing Home and what you know about Wanda Johnson's condition when she was at the University of Louisville, can you think of any medical reason, any medical – medically beneficial reason for a transfer of Wanda Johnson from University of Louisville in the condition that she was on – in on July 24, 1992, can you think of any medical reason, any benefits she could have gotten from that transfer?

A No, not at all.

[p. 20] Q Doctor, one of the diagnosis that I believe that you made once Wanda Johnson was admitted to Midwest Medical Center was that she had, Wanda had active pneumonia; is that correct?

A That's correct.

Q Did you have an opportunity to review the medical records from Crestview and from the University of Louisville to determine whether that pneumonia existed prior to her transfer to Indianapolis?

A When I spoke with the nurse that morning and then I reviewed the records from Crestview Nursing Home, they mentioned that on her arrival that Friday evening that she had a fever of 100.5 axillary, which means under the arm. Which would roughly translate to a temperature of 101.5 if you were to check it by mouth which is what most people are familiar with. They also did mention in their nursing notes that she had abnormal

breath sounds at both lung bases, something that they call rales which can be heard when someone has pneumonia. They also commented on some purulent or sputum that was being produced that looked like it might be infected.

[p. 21] Q Do all of those things indicate active pneumonia?

A Very suspicious for it. Very suspicious for it. There was mention made of a chest x-ray at Humana Hospital on the 23rd after a procedure called a bronchoscopy; unfortunately, the records that I had didn't show that report. I don't know if it actually was done and if it was done I don't – I don't know what the readings were.

Q Doctor, in your practice here in Indianapolis, do you frequently work with the social worker staff at the various hospitals around Indianapolis to place patients after –

A Oh, yes.

Q – they're stable?

A Oh, yes.

Q Tell us, Doctor, what you expect of a social worker when that social worker is going out to – to attempt to place a patient at a – at a new nursing home facility, what do you expect of your social worker to find out about that facility before the transfer is made?

A The social worker will need some medical information when she talks to the nursing [p. 22] facility in order to determine if that facility can meet their medical needs. Initially, they'll break down medical needs into



what we call intermediate level of care versus skilled care which means that you have more nursing time per patient with skilled care than you would with intermediate care and then if they need ancillary services like physical therapy, occupational therapy, speech therapy, whether that facility offers it, whether they're familiar with that type of problem that the patient has.

For example, if I had a patient with Alzheimer's disease and who didn't think clearly, were they familiar with taking care of individuals like that, would they have common sense safeguards in place to make sure that patient didn't get hurt because they didn't think clearly.

So I rely on the social worker to get some information for me, particularly if it's a nursing home or extended care facility that I'm not familiar with. I typically contact and talk with the nursing facility as well when it's one [p. 23] that I'm not familiar with and it's one of my patients.

Q Would you consider it to be outside the standard of good nursing – or good social worker care for -for one of your social workers not to perform those functions that you've just detailed to us when they're attempting to transfer a patient to a new facility?

A She's not doing her job if she doesn't follow through on those things that I mentioned.

Q Tell us, Doctor, if you were supervising a social worker who was transferring a patient with symptoms similar to the symptoms that Wanda Johnson had when she was in Louisville, Kentucky prior to her transfer, what kinds of things would you expect for your social

worker to have inquired about with respect to the nursing home that – that Wanda Johnson was going to be transferred to?

A First, do they offer skilled nursing care; second, have they ever dealt with a patient like this before, have they dealt with people that have undergone significant trauma and have they [p. 24] dealt with – excuse me – successfully in terms of trying to rehabilitate them and with their particular special needs. I think I mentioned before that when I spoke with the facility that morning when I saw the patient they mentioned that this was the first time that they had encountered a patient like Wanda Johnson. They had not dealt in the past with patients with multiple wounds like Wanda Johnson had.

Q So, Doctor, if the social workers at the University of Louisville Hospital failed to check into those aspects of the services that could be provided by Crestview Nursing Home, do you have an opinion as to whether their activities were within the standard of good social worker care?

A I don't think they had the best interest of Wanda Johnson at heart. I should mention that I confirmed this by talking to the medical director of the facility to see if indeed they had dealt with patients like this in the past and they had not.

Q You're talking about the medical director at Crestview Nursing Home?

[p. 25] A That's correct. That's correct.

Q So let me make sure I understand your comment. You're saying that if the social worker at Humana didn't



check into these things then you don't think that she had the best interest of Wanda Johnson at heart?

A No, I don't.

Q Doctor, you indicated that one of your partners was at least on July 24th and '5th of 1992 medical director at Crestview Nursing Home?

A That's right.

Q Do you recall – first of all, is your medical group the medical directors for that facility today?

A No, we're not.

Q And when did you cease providing that service to this particular nursing home group approximately?

A I'm not sure of the actual date. I would say sometime within two months but that – that I don't remember specifically, but probably within two months of her arrival at the nursing home, perhaps three.

Q Did your medical group's decision or the mutual [p. 26] decision between your medical group and the Crestview facilities to part ways have anything to do with their acceptance of Wanda Johnson in this particular case?

A I didn't directly speak with the administrator particular – specifically about that. I did speak with the physician who was supposed to be responsible for her and so what I received was secondhand information from him that this was one reason why we were no longer involved with their facility because of complaints or objections brought up by doctors in our group; but again, I didn't hear this specifically from the facility, this was just told to me by other physicians who attended there.

Q Do you or members of your medical group accept referrals from Crestview today?

A I'm not sure I follow you, accept referrals?

Q Okay.

A They typically have a physician assigned to them, usually the medical director or they might have private physicians that follow them at the nursing home so those physicians would be [p. 27] following those patients if they went to the hospital. I can't myself recollect seeing a patient from Crestview Nursing Home for quite some time. I have taken care of one other patient from Crestview Nursing Home who also arrived in the emergency room and I happened to be on call, but it's been quite some time, well over a year since I've seen any residents from Crestview Nursing Home.

Q Doctor, could you tell us a little bit about what injuries, if any, or what deterioration in Wanda Johnson's condition you noticed as having been caused by her transfer from the University of Louisville Hospital?

A When I met her Saturday morning, she had a very high fever, she was breathing very rapidly, clinically she demonstrated evidence for pneumonia and by x-ray she also demonstrated evidence of pneumonia. Her white blood cell count was high. She was flushed, sweaty, hot. I felt she demonstrated all the physical findings and laboratory findings consistent with early sepsis or overwhelming infection.

[p. 28] Q And –

A If this is not treated, then it results in death.



Q Doctor, was Wanda Johnson's condition substantially worse when you saw her on Saturday morning than what it appeared to have been just prior to her transfer from the University of Louisville Hospital?

A Her res – yes, based on the notes that I read, her – her respiratory rate was very rapid, her pulse was very rapid, her temperature was 105 at the nursing home just prior to transfer to the hospital which is higher than at the hospital – Humana Hospital; so yes, she was worse than – than at Humana Hospital.

Q And did the fact that she was transferred from that hospital to Indianapolis have anything to do with the deterioration in her condition in your opinion?

A It's not so much where she was, it's the treatment that she needed. If she had remained in the hospital, I'm sure she would have demonstrated the same problem and then it would have been treated there. The risk or the danger [p. 29] is that she was outside the hospital for a period of time as she got worse and not immediate – and not directly under the care of a physician that could take care of her quickly. So, you know, there is a window of time when she was transferred from the hospital before she arrived in the hospital. I think it was about eighteen hours between arrival at the nursing home and when she was in the emergency room, and during that time, she deteriorated significantly.

Q Doctor, had she remained in a hospital setting and had medical attention available to her, is it probable that this episode of – of deterioration in her condition could have been avoided?

MR. THOMPSON: Objection to the form.

Q Doctor, in your opinion, could the deterioration, the rapid deterioration in Wanda Johnson's condition which you've described to us, in your opinion, could that have been avoided had she been in a hospital rather than a nursing home?

A She probably – well, I'm sure they would have noticed the deterioration in her condition and [p. 30] she probably wouldn't have gotten quite as far along because they would have intervened more quickly and addressed her problems. She's had recurrent pneumonia in the hospital at Humana before and – and I'm sure they would have noticed the elevated temperature and addressed her problems.

Q Would it be a fair statement, Doctor, that the further the – the more significant the digression in her condition the longer it would take the hospital, whether it was Midwest or whether it was Humana, to bring Wanda Johnson back to the condition she was prior to that digression?

MR. THOMPSON: Objection. Form.

A Can you run that question by me again.

Q Sure! Doctor, in your opinion, would the degree that Wanda Johnson's condition deteriorated –

A Uh-huh.

Q – have anything to do with how long it took to – to treat Wanda Johnson and bring her back to a stable position – condition?



A She was started on an antibiotic that did not [p. 31] address what was cultured from her urine and what was ultimately cultured or grown from her sputum. So during the time that she was on the antibiotic that was started at Humana Hospital until the antibiotic therapy she received at Midwest, the infection that she had was not being treated successfully and the fact that she had such a high fever and had the signs and symptoms of early sepsis just reflect the infection getting worse and worse.

If another day had gone by, I – she might have passed away if she had remained in the nursing home still on that same antibiotic, tough to tell. It varies a little bit from person to person how quickly that can occur. In her case, because of the fact that she had a catheter in her bladder which makes you more prone to infection, the fact that she didn't have a spleen which helps fight off certain types of infection, she's at higher risk for infection and at higher risk for deteriorating more quickly.

Q Doctor, in your opinion, does the degree of family support that a patient in Wanda Johnson's

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UNITED STATES DISTRICT COURT
WESTERN DISTRICT OF KENTUCKY
LOUISVILLE DIVISION
CAUSE NO. C-93-0544L-M

[Caption Omitted in Printing]

The deposition upon oral examination of KAMBIZ T. KARIMI, M.D., F.A.C.P., * * * taken on the 12th day of July, 1994, * * *

* * *

[p. 12] Q. Did you have any involvement as the medical director with the state regulatory agencies that oversaw nursing homes in the state? Did you ever meet with state regulators?

A. Uhn-uh. State regulators on this case or any?

Q. No, sir.

A. At anytime at all?

Q. As the medical director, I'm just asking whether part of your job was to prepare reports.

A. No, no, no. Part of our job was to meet every, I believe at that time it was every month, or have to look and see. By law we meet on a regular basis. It used to be every month, and now it's changed to every three months, quality assurance. Now it's just every three months, some places require every three months to discuss any type of policy procedures, problems, address the



problems. That's routine for all the nursing homes. They have to do it.

Q. Let me ask you a couple of questions about Wanda Johnson, You never treated Wanda Johnson, did you?

A. Never seen her. Never knew she was coming. I was the one who sent her out.

Q. And there is a note here that says on July 25, 1992, [p. 13] at 9:10 a.m., "Call placed to Dr. Karimi," and then you ordered her transferred out. Wanda Johnson, when she arrived at Crewstview [sic] she had a tracheostomy, and Crestview had the capabi1ity of caring for a patient with a tracheostomy, did it not?

A. Tracheostomy, it is not a very difficult thing to take care of. But they have a nursing staff take care of, and they were – at one time all nursing homes began to have so-called skilled facility to provide care for the patient with much more skilled case.

Were they able to take – to be honest with you, I don't know whether they were able to take care of it or not. I reviewed the chart. I read the chart after I heard there was some of this, and I just wanted to make sure what was my involvement in the case. And prospectively, they did not inform us about admission. Retrospectively, I think probably that was an inappropriate admission.

Q. At Crestview?

A. Yes.

Q. Are you saying that because you believe Crestview was not capable of providing the care Wanda Johnson [p. 14] needed?

A. Yes, knowing the patient now what I know about her. Again, I never saw the patient, nothing. They called me, said the patient's fever is 104. I assigned the patient immediately to one of our physicians. And she stayed in the hospital what, twelve hours, sixteen hours at most in there. And our policy is to see the patient within 24 hours skilled, and 48 hours nonskilled, and we send them out.

And now what I know about the patient, probably it was an inappropriate transfer.

Q. Why was it inappropriate for Crestview to admit Wanda Johnson or to accept her as a patient?

A. Well, there was a – I don't know. I didn't see documentation. I didn't know but they said she was febrile [sic] even on the way coming to Crestview. But then the physician, Dr. Shatara, said the patient was 100.5.

Q. Is that febrile?

A. Axillary, it probably is, but axillary. If it is oral, even if it's a fever, it's not that significant. Even as fever, 100.5, we send people home with 100.5. We send people back to nursing [p. 15] homes with 100.5.

Going back again to the tracheostomy, I will just have to guess that they have the nursing staff, they did have people who they thought they can take care of patients.

Q. Take care of patients with a tracheostomy?



A. Yes. I mean, it's not that difficult to take care of a patient with a tracheostomy.

Q. There were skilled nurses at Crestview, were there not?

A. I'm sure, when you are an RN. Just like me, I'm an M.D. I can do neurosurgery if I want to, but if I mess it up, I'm grilled. An RN, they can do anything they want as long as they have experience.

Q. Well, would you agree with me that Crestview should not have accepted a patient with a tracheostomy if they did not have a nurse trained to care for that patient?

A. Sure, definitely.

Q. And Crestview, the administrators at Crestview would be the people who should know what the capabilities of their nurses are; correct?

A. Oh, yes, definitely.

Q. [p. 16] A patient with a urinary tract infection can be treated on an outpatient basis; correct?

A. Yes.

Q. Patients in nursing homes get urinary tract infections?

A. All the time.

Q. You treat them there. You don't move them to a hospital, do you?

A. No. As a matter of fact, these days now there is so much changes that most of the patients, almost all patients are treated at the nursing home unless they are

septic. That means they are what we call the blood, the bacteria has gone to the blood. Even then we decide, we have made a decision whether we should transfer the patient, should we treat the patient in the nursing home. So not everybody goes to the hospital.

Q. You discharge patients from hospitals into nursing homes yourself?

A. Have I discharged patients from a hospital to the nursing home?

Q. To the nursing home.

A. Yes.

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UNITED STATES DISTRICT COURT
WESTERN DISTRICT OF KENTUCKY
LOUISVILLE DIVISION
CAUSE NO. C-93-0544L-M

[Caption Omitted In Printing]

• • •

The deposition upon oral examination of Taiseer Shatara, M.D., • • • taken on the 12th day of July, 1994 • • •

• • •

[p. 17] you for any reason based upon information you got from the nurse felt that that patient's healthcare was seriously endangered –

A. At that time I felt –

Q. – at that time you would have transferred her?

A. At that time, yes. But at that time the information, and I have to trust the person on the line that gave me the appropriate information.

Q. Okay. Doctor, you've told me that, that you need to trust the person on the end of the line.

A. Yes, yes.

Q. And I accept that. I'm asking a different question now. And that is that if you didn't feel comfortable with the information you got over the telephone, I assume you would have taken some steps to act on that?

A. Well, again, based on the information that was given to me at the time, I felt that the patient was stable enough.

Q. Okay.

A. At that time.

Q. Okay. Let me hand you a copy of a note that's dated and timed – it's dated, rather, July 24, 1992. Have

* * *

[p. 19] admission, would you look at those, please.

A. Yes.

Q. Do those appear –

A. That would be reasonable.

Q. – appear to be reasonable?

A. A patient who has been transported, yes.

Q. There is certainly nothing contained in the information in these notes that would indicate this patient is unstable or could not be cared for?

A. Not on the basis of this.

Q. You were aware at the time of Ms. Johnson's admission that she had a tracheostomy and a Foley catheter and an N/G tube?

A. I was told she had a tracheostomy. I don't remember the N/G tube. I remember she told me about the wound. That why I asked about the wound, and she stated it was as she stated in her note.

Q. Based upon your experience at Crestview, you were comfortable that they could care for a patient with a tracheostomy?



A. Yes.

Q. Dr. Shatara, Crestview had the capability, did it not, to obtain lab specimens, such as urine cultures [p. 20] and blood cultures?

A. Uh-huh, yes.

Q. And it was capable of managing a patient who had a urinary tract infection?

A. Yes.

Q. If a patient at Crestview was not responding to prescribed antibiotics for a urinary tract infection, could antibiotic therapy be modified in the nursing home setting?

A. In certain nursing homes, yes.

Q. At Crestview?

A. Yes.

Q. Is it fair to say that if Wanda Johnson had arrived at Crestview and you had received information to indicate she needed to be in a hospital, that you would have taken the necessary steps to transfer her?

A. Yes.

Q. Doctor, finally, is it fair to say that when you admitted Wanda Johnson to Crestview, based upon the information you received from Crestview, Wanda Johnson was stable and was an appropriate nursing home placement?

A. Now, repeat that question again.

[p. 21] Q. Would it be fair to say that at the time you admitted Wanda Johnson, would it be fair to say at the time you admitted Wanda Johnson to Crestview, based upon the information you received from Crestview's nurses, that she was stable and that she was an appropriate candidate for nursing home placement?

A. At that time, yes.

MR. THOMPSON: I want to go ahead and mark as Exhibit 2 the nurse's notes from Crestview.

(Defendant's Deposition Exhibit No. 2 was marked for identification)

Q. Doctor, there is a reference at the 6:00 p.m. entry regarding your discussion with the nurse, and it says that the nurse received an order to call the resident's family on res. code. Does that refer to resuscitating code?

A. Yes.

Q. Is that standard?

A. I usually do that with every patient, yes.

Q. Every patient that comes in there?

A. Yes, so this is not – at the hospital or the nursing home.

Q. Did you have any further contact with Crestview after

* * *

[p. 23] A. Not really. Just, you know, the initial information that this patient had a tracheostomy and open wound, but I did not get a detailed history that this



patient had other problems until later on after she was in the hospital.

Q. How did you learn of those other problems or that other history?

A. After the patient was hospitalized.

Q. And did you obtain that information by reviewing her records or by talking to other physicians?

A. By talking to other physicians. In fact, I did the cross-coverage for the physician who admitted her into the hospital.

Q. What significant history or other problems did you discover that you are referring to here.

A. If I had known that this patient had the complicated hospital course that she had, I would not have accepted the patient to be in a nursing home.

Q. And when you say the complicated hospitalization that she had, specifically is there anything that you are referring to?

A. From multiple infection to the complicated course requiring multiple surgery to intervene with certain [p. 24] infection, intra-abdominal infection that she had, and the need for her medical care that she required much more so than just being in a nursing home.

Q. Doctor, I know that you indicated that you don't recall the specifics of the conversation that was held between you and the nurse at Crestview at the time when Wanda was admitted. Do you recall specifically during that conversation whether or not anyone told you that

Wanda, that it was suspected that Wanda may have had a urinary tract infection?

A. Not really.

Q. So that information did not come to you?

A. No, that information did not come.

Q. Do you recall whether or not it was conveyed to you at that time that Wanda Johnson's white blood count had been elevated or was elevated?

A. No, not at that time.

Q. You were not advised at that time?

A. I was not advised of her white blood cell count.

Q. Based on your experience with patients who are transferred from a hospital to Crestview or any other nursing home, would it be a typical situation that the transferring hospital would provide much of the [p. 25] details of the patient's history to the nursing staff at the hospital prior to the time that the nursing staff contacts you?

A. Usually is the case you are provided with a discharge summary from the hospital.

Q. Do you have any knowledge or information about how much information was provided by the University of Louisville Hospital to Crestview nursing home or to the nurse who made the telephone call to you when Wanda Johnson was admitted?

A. Not really.

Q. Doctor, had you known about the complicated hospitalization that Wanda had experienced that you



described to us, had you known about the suspected urinary tract infection, had you known about the elevated white blood count that Wanda had in addition to the other information that you say you do recall the nurse providing to you on the evening of Wanda's admission to the nursing home, would you have done anything differently?

MR. THOMPSON: Objection to form.

A. I would not have accepted her.

Q. Doctor, when you say you wouldn't have accepted her, [p. 26] what do you mean?

A. I would probably have had her transferred to a hospital.

Q. Immediately?

A. Yes.

MR. MATTINGLY: Thank you, Doctor. That's all.

EXAMINATION,

QUESTIONS BY MR. THOMPSON:

Q. Doctor, a few more questions. I believe you just testified that you expected the nurse at the receiving facility at the nursing home to have a discharge summary; correct?

A. Yes. I wasn't told what she had. Sometimes –

Q. You've answered my question. Let me go ahead. Doctor, did you ask the nurse to read you –

A. I don't remember.

Q. Excuse me. Let me finish. Did you ask the nurse to read you the discharge summary from the University of Louisville Hospital?

A. I don't remember.

Q. You could have? You don't remember?

A. I don't remember. But let me – many times a patient

* * *

[p. 37] received some compensation for the work you did at Crestview?

A. Yes.

Q. Had you taken care of patients at Crestview prior to July of 1992 who had had a tracheostomy in place?

A. I really don't remember.

Q. Is that significant to you one way or the other?

A. No. I've taken care of since then. It really is not.

Q. At Crestview?

A. No, not at Crestview.

Q. At a different nursing home?

A. At a different nursing home.

MR. THOMPSON: I think that's all I have. Thank you.

EXAMINATION,

QUESTIONS BY MR. MATTINGLY:

Q. Doctor, just a couple of follow-up questions. The orders that you gave to the nurses at Crestview when you



talked to them the night of Wanda Johnson's admission concerning taking the vital signs and the frequency of those vital signs being taken, were those orders, whatever they were, based on the [p. 38] information that the nurse had provided to you at the time when she called you at 6:00 p.m. on that night?

A. Yes, sir.

Q. And, Doctor, would it be a fair statement to say that there was extremely important medical information about Wanda Johnson that was not made available to you on the night of her admission?

MR. THOMPSON: Excuse me. I object to the form. He's already testified he doesn't know whether he reviewed the discharge summary or not, so I object to it.

Q. Let me ask the question again: Doctor, would it be a fair statement to say that now, based on your knowledge of Wanda Johnson's case, that there was extremely important medical information about Wanda Johnson's condition that was not made available to you during that initial telephone call from Crestview to you the night of Wanda's admission?

MR. THOMPSON: Objection to the form.

A. Yes.

Q. Your answer again?

A. Yes.

MR. MATTINGLY: Okay. That's all.

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UNITED STATES DISTRICT COURT
FOR THE WESTERN DISTRICT OF KENTUCKY
LOUISVILLE DIVISION
CIVIL ACTION NO.: C-93-0544L-S

[Caption Omitted In Printing]

* * *

The deposition of FRANK MILLER, MD, taken pursuant to notice by the Plaintiff on January 25, 1995, * * *

* * *

[p. 14] Q. Typically, Doctor, when a – when a policy is – is revised, is – is that revised policy circulated among the residents and the – the doctors in the – in the hospital?

A Generally not, no.

Q How would – how would a – a – a resident or a –, a doctor on staff at the hospital know that a particular policy has been changed?

A By going to the various committees and – and then by the people like myself informing them.

Q All right.

A You know, so it's part of my job. If something that I think they need to know about has been changed or revised or – then – then we would let them know.

Q Okay.

A But generally these type of things, if there's something that they really need to know about, then you might send it around to all the residents.

Q Okay.



A You know.

Q Let me ask you, Doctor, these two policies, one number 418 that indicates its [p. 15] discharge criteria from a critical care complex, number 420 indicates the subject is transfer of patients from the critical care complex to another facility, would – would these two policies be applicable to all discharges and transfers from the critical care complex at the University of Louisville Hospital?

A Yes. Now, that doesn't mean that every single thing is followed because, again, these are policy procedures which are general guidelines and – and – but – but they would be applicable, yes, to all the patients.

Q All right. Let me refer you to – to policy number 420 at – at this point. And if Wanda Johnson was transferred from the critical care complex on July 24 of 1992, would this policy have applied to – to her transfer at that time?

A Now, that – run that question by me again?

Q All right. If Wanda Johnson was transferred from the critical care complex –

A (Interrupting) Uh-huh.

Q (Continuing) – of the University of Louisville Hospital on June 24th of 1992 to another facility, would policy number 420 have been [p. 16] applicable to her transfer?

A Well, I assume it would if it was – already been approved and in place.

Q All right. In the fourth paragraph on that policy, the policy indicates that transfers can take place only after the transferring physician has received approval for the transfer by the patient or the patient's family and the accepting physician. As to the accepting physician, is it policy within the University of Louisville Hospital for – for the – the patient's physician at U of L to contact the physician at the other facility to receive acceptance of that patient?

A No, not necessarily. A lot of this is done – a lot of this is done through nursing and discharge planning, so it's not necessarily that we always talk with the referring physician. As a matter of fact, most of the time when I transfer patients, say, from University or Norton's or Jewish to Frazier, I will not personally have talked with the referring physician. Now, someone has and they are – you know, they are then about the patient, but it's not necessarily a – a physician-to-physician contact. It may be, it depends on the situation.

[p. 17] Q Now, you've been referring to – you've been saying the referring physician. Are we talking about the physician who's going to be treating the patient at the receiving facility?

A Well, that would be the – the – no, the referring physician would be the patient – the – the physician who's sending the patient. So if I were the referring physician, I may or may not talk with – directly with the physician to whom I'm going to send the patient. I might, but I might not.

Q All right.



MR. THOMPSON: Let me make sure – I think we're getting confused here and I want to make sure everybody's talking apples and apples and not apples and oranges. You're talking about transferring a patient to –

A (Interrupting) Another facility.

MR. THOMPSON: For critical care or acute care or to a skilled nursing home?

A Either – either one. If it's a skilled nursing home, then I almost never talk with the – almost never talk with the physician that's going to take care of the patient.

Q Okay. The policy refers to the [p. 18] accepting physician and I assume that's – we're –

A (Interrupting) Right.

Q Who we're talking about here –

A (Interrupting) Right.

Q (Continuing) – at this receiving facility?

A Uh-huh.

Q Even though a contact between the physician at U of L and the accepting physician is not always made physician to physician, is the policy that someone is supposed to contact the accepting physician to have that patient accepted? Is that what this policy says?

A Well, to be honest, I mean, I – pol – there's policies for everything and so I sure – I read these, but I sure don't memorize them and know them all by heart. So –

MR. THOMPSON: (Interrupting) Doctor, why don't you take a minute and read that carefully before you testify about this?

A Yes. Right. Transfer – let's see. Fourth one, transfers will be made using good medical practice and for the benefit of the patient. No patient – patient – wait.

MR. THOMPSON: Excuse me, Doctor.

[p. 19] Q Fifth. Fifth.

MR. THOMPSON: Would you mind – A (Interrupting) I'm sorry, it's the next one.

MR. THOMPSON: Would you mind just starting at the top and read to yourself and make sure you understood what this applies to?

A Okay. Okay.

Q All right. My question, again, is – is in reference to the fifth paragraph –

A (Interrupting) Uh-huh.

Q (Continuing) – on this particular policy. And my question to you is: Would it be consistent with the policy of the University of Louisville as evidenced by policy number 420 that when a patient is transferred from a critical care complex to another facility that the physician or someone on the staff is to contact the accepting position – physician to verify his acceptance of the patient?

A Generally that would be – that would be true and it would – it would depend a lot on the condition of the patient and where the patient was going. If the patient is going to a hospital, then you – almost always someone



would contact – would [p. 20] contact that – would contact that – that physician who's going to be taking care of that – that patient. If the patient is going to a rehab center or if the patient's going to a nursing home, then generally we would not. Now, most of the time when patients – the majority of the time when patients go from our facility to a nursing home or a rehab center, they're not coming from a critical care complex but occasionally they do. So according to this policy, what it says is that – that the – that they would – they have received approval by the accepting physician, but that doesn't mean that we would necessarily make contact or talk with them. We would sure have to make contact or talk with someone in that facility that's – that sort of takes the authority and the responsibility for saying yes, our facility can take care of this patient.

So we would – we would assume unle – if we did not think we needed physician-to-physician contact or directly to talk with that physician, that – that's been done by the – by the person, whoever it is that we contact. And usually most of the time this is done by social service and [p. 21] discharge planning and – and not by the physician.

Q All right. You've indicated that – that you – someone talks to the facility, but again, reviewing paragraph five there, doesn't it indicate that the policy is that – that someone at least has to receive approval from the accepting physician?

A Right. And we're assuming that – that their representative that we deal with from that facility has done that.

Q All right.

A I mean, we can't – if – if the patient – again, it depends on the patient. It's – it's a judgment decision. If the judgment is that, boy, this is not good enough and we need to talk with the doctor – doctor about it or it may be that, boy, this isn't good enough and our doctors need to talk with their doctors. So it all depends on – it all depends on – on the individual patient.

Q All right. The next paragraph in that policy indicates that the critical care complex will secure a written, informed consent prior to transfer, and then lists some things that the – that the consent is to – is to include. Is that

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[p. 27] Q Would you read the second paragraph of the purpose for policy 420, rule 420?

A To assure that all patients who have an emergency medical condition are transferred in accordance with the COBRA requirements.

Q Does this policy, then, apply to patients who, in fact, have an emergency medical condition?

A Yes.

Q Would it be unusual or inappropriate – strike that. Would it be inappropriate to transfer a patient who has an emergency medical condition to a nursing home?

A No, it would not be appropriate.



Q Is it your opinion, do you have an opinion whether this policy would apply to patients who are being discharged in a stable condition to a nursing home?

A I –

MR. MATTINGLY: (Interrupting) Before you answer, Doctor, let me object to his opinion rendered about Wanda Johnson or a patient in general using the terminology that you're using, because I think all of those are – are – are legal terms rather than medical at this point, and Doctor Miller [p. 28] has not been identified as an expert witness to provide his opinions in this case as to the appropriateness of a transfer.

Q Doctor, you – you did rounds on this patient, Wanda Johnson, did you not?

A Yes. I saw her several times in ICU.

Q The residents, medical students who were taking care of her, were they under your supervision?

A To some extent, yes.

Q You were the chief of the –

MR. TANNER: (Interrupting) Intensive care.

Q You were chief of surgery –

A (Interrupting) Yes.

Q (Continuing) – correct? This was a surgical patient?

A Yes.

Q You were also the chief of the surgical intensive care unit where she was a patient?

A Right. Yes.

Q All right. And you sat on the committee that was involved in promulgating policy 0420, correct, sir?

[p. 29] A Yes.

Q For the – for the patient, Wanda Johnson, who was being cared for by your students and residents on the floor where you were the acting chief, did this policy apply – would it have applied to Wanda Johnson?

MR. MATTINGLY: Objection. Question's already been asked and answered.

Q Go ahead.

A Well, it – I think that's a matter of opinion. These are general guidelines for patients in our critical care units and – but – but – so as – in terms of that she was in our critical care unit. So the guidelines would – would – would apply as general guidelines, but to say that everything here applies to Wanda Johnson, almost – medicine is very complex and there's hardly any patient where you can write a guideline where the guidelines always apply to that patient. I mean, they're guidelines.

Q And this policy applies to patients in the critical care complex?

A Yes.

Q And it applies to those patients who have an emergency medical condition, correct?

[p. 30] A Yes.

Q Did she on July 24, 1992, based upon your review of the record, have an emergency medical condition?



MR. MATTINGLY: Objection.

A In my opinion –

MR. MATTINGLY: (Interrupting) Let – let me – I'm gonna let you answer, but let me object again, because he has not been identified as an expert. The time for identifying experts has already passed. He's – he's here as a fact witness only and – and we would object to the giving of any opinions concerning Mrs. Wanda Johnson's condition.

Q Go ahead, Doctor.

MR. MATTINGLY: Go ahead and answer.

A In my opinion, she did not have an emergency medical condition on the date of her trans'er.

Q Doctor, did – did the physician who discharged Wanda Johnson on July 24, 1992, in your review of the records, was that discharge appropriate?

A Yes.

Q Doctor, in – in your role as a chief of the surgical intensive care unit and chief of [p. 31] department of surgery, if a patient was discharged from this facility who – who should not have been char – discharged based upon the patient's medical condition but was discharged solely because of a financial reason, would that be brought to your attention?

A Well, I would hope so. And I would – I would say yes. I have never known that to have happened, but I would assume and hope that it would be brought to my attention, yes.

Q Based upon your knowledge of this patient and your review of the records and your – and your position as a chief of department of surgery and chief of the surgical intensive care unit, are you aware whether this patient was discharged or whether her financial circumstances played any role at all in her discharge from this facility?

A Well, the on – I can say something as a general policy. Basically the attendings and the residents never have any idea what the financial condition of the patient is. Now, we almost never know that and, as a matter of fact, for most of our trauma patients, the hospital really doesn't even know it, especially because so much of their – [p. 32] their reimbursement really is an unsettled issue for months and months because so many of 'em are accidents and injuries. And especially motor vehicle accidents, where you don't know even if the patient has no money or no insurance. Many times three months later, six months later you find out that the other party has insurance that covers them or – or – or it is gonna be work related or there's a lawsuit involved in terms of negligence and creating the injury and somebody else pays for it.

So – so we – as an attending physician and – and as a – as a resident, we almost never find out who has – who has insurance and who doesn't and – and – and what the pay status is.

Q And certainly, at least to this date, no one's made you aware that Wanda Johnson's discharge from this hospital was motivated in any way by her financial circumstances?



A I cannot imagine that it was.

Q Okay. But my question was: Has anyone ever made you aware of that?

A No, no one's ever made me aware of that.

[p. 33] Q No resident or attending's come to you, said, "Doctor, the hospital is telling me to get this patient out of here because she doesn't have money to pay for her care"?

A The hospital never does that. Not this hospital.

Q Go back briefly to policy 0420 and I'm just about finished here. If a patient was sub – had an emergency medical condition and was being transferred from this hospital to another facility, I take it it would be important that there be communication with the physician who's going to be rendering care for that emergency medical condition?

A Yes.

Q And is that basically what this policy's dealing with, Doctor?

A Well, I think that's a lot to do with how – what this policy is dealing with, yes.

MR. THOMPSON: Anything else? (Whereupon Mr. Thompson and Ms. Tanner conferred out of the hearing of the reporter.)

MR. THOMPSON: Doctor, I think that's all I have.

MR. MATTINGLY: Doctor, just a couple follow-up questions.

[p. 34] REEXAMINATION

By Mr. Mattingly:

Q Referring again to policy 0420, Mr. Thompson has directed your attention to the second paragraph of the purpose statements on that policy, but there is a second purpose identified there also, is there not?

A Yes.

Q All right. And that purpose indicates that the policy is to provide guidelines for the safe and efficient transfer of patients from the critical care complex to another facility. Isn't that the second purpose for this policy?

A That's another purpose.

Q All right. You testified earlier that in all likelihood, you were on vacation at the time that Mrs. Johnson was discharged from the hospital; is that true?

A Yes.

Q And other than possibly a review of Mrs. Johnson's medical records, you had no involvement in the discharge plan or the actual discharging of this patient, did you?

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UNITED STATES DISTRICT COURT
FOR THE
WESTERN DISTRICT OF KENTUCKY
LOUSIVILLE DIVISION

CAUSE NO. C-93-0544-L-M

[Caption Omitted In Printing]

* * *

The deposition upon oral examination of **JOHN W. STUY, M.D.**, * * * on March 28, 1995, * * *

* * *

[p. 13] (EXAMINATION – BY MR. THOMPSON)

The bronchoscope can be very helpful, too, in trying to clear areas of blockage so that someone can clear secretions better. You can use it also to obtain cultures to help guide your therapy, if someone isn't responding to therapy.

Q. And are you aware of, do you know whether there were earlier chest X-rays taken?

A. She had several chest X-rays taken during her stay at Humana.

Q. Are you able to say as of July 23, 1992, the day of the first of the two reports that were provided to you, where it says that the right upper lobe atelectasis and/or atelectasis, which you think was infiltrate –

A. Right. That's what they meant.

Q. – do you know to what degree it had improved by this point?

A. They don't comment on it. I looked at the films myself, and I mean, I think I see some subtle improvement. I

would be hard pressed to quantitate it. I would have to put it [p. 14] back up on screen and try to quantitate it in terms of percent, if that's what you are looking for.

Off the top of my head, I wouldn't want to speculate without looking at the films. But I agree with the report. I did think there was some improvement.

Q. There was some improvement from the July 24 film compared to the July 23 film; correct?

A. Right.

Q. When you say some improvement, what are you comparing it to?

A. What I'm looking for is the size of the infiltrate and/or atelectasis. And it appeared to be a little bit smaller on the 24th compared to the 23rd. That's what I meant, and I assume that's what they meant as well.

Q. Any other materials that you were provided since your January 25, 1994 deposition?

A. I received a copy of my deposition, which I remember skimming over, uh-huh.

Q. Anything else?

* * *

[p. 23] platelets more than the white blood cells that would be affected.

Q. You have discharged patients from hospitals with a urinary tract infection, either to home or to a skilled nursing facility; right?

A. Yes.



Q. In discharging a patient home with a urinary tract infection, what type of treatment do you prescribe for that?

A. Typically an antibiotic, something perhaps for the discomfort, and some instructions, and usually a follow-up urinalysis to be sure that the infection is clear a few weeks after therapy has been completed.

Q. If a patient were being transferred or discharged to a skilled nursing facility, the same type of medications you would expect to be given to them, and would there be some follow-up at the skilled nursing facility?

A. Most all nursing homes that I'm aware of, if a patient arrives on any sort of an [p. 24] antibiotic therapy, wi11 have their blood pressure, pulse and temperature checked at least every shift. And usually, again, you would ask for a follow-up urinalysis after completion of therapy. And most nursing homes would ask for that if the transferring doctor didn't ask for that himself.

Q. Skilled nursing facilities, you would expect them to have personnel trained who could take vital signs; correct?

A. Of course.

Q. You would expect the nurses at a skilled nursing facility to know to call a patient's physician if signs and symptoms are found that are indicative of a deterioration of the patient's condition?

A. I would expect that, yes.

Q. Do skilled nursing facilities in the Indianapolis area, has it been your experience that they have the

capability to obtain urine cultures or other lab work for patients if ordered by a patient's physician?

[p. 25] A. Yes.

Q. Would that be true for Crestview Nursing Home in 1992 to your knowledge?

A. I've never worked with Crestview, but I would assume that they could do that. Otherwise, they wouldn't remain accredited.

Q. Who accredits nursing homes, by the way?

A. The state will inspect nursing homes. There are state and federal guidelines that have to be met.

Q. Do you know which branch or agency of the state licenses nursing homes?

A. Gosh, I don't remember offhand, no.

Q. Is there a health board or something?

A. Yes.

Q. Is that different from the agency that would license nursing homes, or do you know?

A. I don't know, either.

Q. Would you expect, back to the urinary tract infection, would you expect that a skilled nursing facility would be able to manage and care for a urinary tract infection?

A. Yes.

* * *



[p. 28] brought to my attention a few months ago.

Q. You can be a patient who is eligible for Medicaid benefits and have requirements of skilled nursing care, and those requirements aren't lessened at a nursing home just because a patient was Medicaid versus Medicare?

A. They are not supposed to be.

Q. I'm skipping around on you a bit here. I apologize for that.

A. That's all right.

Q. In terms of this urinary tract infection we were discussing a moment ago, pending the results of a culture and sensitivity test, broad spectrum antibiotics typically are initiated for the patient pending those results; is that correct?

A. You would pick an antibiotic to cover the expected organisms, that's right.

Q. We've already talked earlier and said the antibiotic they placed Wanda Johnson on at the time of her discharge was a reasonable antibiotic?

[p. 29] A. In my opinion, yes.

Q. If you discharged a patient from your hospital on a broad spectrum antibiotic, awaiting the culture and sensitivity report, and you got that report back, and it indicated that the causative organism was resistant to the antibiotic you prescribed, would you simply change that patient's prescription?

A. I would contact the facility, yes, and change her antibiotic.

Q. Yes. You wouldn't have her brought back up and examine her. You would simply change the medication she was on to an antibiotic that would address that organism?

A. Assuming she is not having difficulty, yes.

Q. Right, assuming she's not having difficulty. If Ms. Johnson had remained at Crestview after her initial transfer there and had not had a complication that required her to come to the emergency department – are you with me so far?

A. Uh-huh.

• • •

[p. 32] speculate what I think was the source of it, but I don't know for a fact.

Q. Well, let me ask you, have you reviewed the records at Crestview?

A. Yes, uh-huh.

Q. Do you have an opinion within a reasonable degree of medical probability as to what caused her condition to deteriorate from the time of discharge until she was transferred?

A. Either the urinary tract infection that she was treated for was not covered, was not sensitive to the antibiotic that she was sent on transfer, or she developed another acute pulmonary infection.

Q. Have you reviewed the nursing care that was rendered to Ms. Johnson at Crestview in terms of whether she was appropriately –



A. I reviewed the nursing notes, yes. You showed me those notes, also, at the first deposition.

Q. Right. Do you have an opinion whether Ms. Johnson was adequately monitored by the nurses at Crestview Nursing Home?

• • •

[p. 61] home may need to manage the urinary tract infection?

A. Or pneumonia.

Q. Or pneumonia because both of those had been recurring throughout her hospitalization at Midwest; correct?

A. That's correct.

Q. Assuming you were satisfied with the nursing home and its ability to handle Wanda Johnson, you were going to transfer her, knowing that she could develop a urinary tract infection or pneumonia while in that facility?

A. That's correct. If they are capable of managing it, then I would transfer.

Q. Right. I understand. But just so I'm clear on this, just because she had a risk for the, a high risk for urinary tract infection and pneumonia, that did not mean that she needed to remain in your hospital or some other acute care facility?

A. That's correct.

Q. I'm getting close to the end here, if that

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[p. 79] problems." And she said, "Yes. No, we've never taken care of someone this complex at this facility."

That's what I was told. And again, I know you had asked me this before, I did not have the name of the nurse that I spoke to. But again, there were only so many nurses on duty on a Saturday morning; so you should be able to sort that out.

Q. This is a nurse at Crestview?

A. Right. I tried to get some information about the patient from the facility.

Q. Would you expect a nursing home, if it had never taken care of a patient such as Wanda Johnson, would you have expected Crestview to be up front about that with the social workers at the University of Louisville Hospital?

A. Yes.

Q. And tell them, We've never done this before. We've never had a patient this complex in our nursing home.

A. I would expect them to say so, yes. I would [p. 80] expect the social worker at Humana to ask those questions, too.

Q. You would expect to get truthful answers from the representative of Crestview; correct, sir?

A. That's correct.

Q. To the extent it is a licensed facility, to the extent it is certified by the state, you would expect you would



not have a problem if a social worker got answers from that nursing home and relied on those answers?

A. No, I wouldn't have a problem if they felt they were getting truthful answers. What else can you do when you talk to people?

Q. Okay. And you would certainly expect that if a nurse had gone to the administration of that nursing home and said, "I do not think we can take care of this patient," that a decision to accept that patient because the facility wants the revenues, that that would be egregious?

A. Terrible.

Q. Terrible. Thank you.

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UNITED STATES DISTRICT COURT
WESTERN DISTRICT OF KENTUCKY
LOUISVILLE DIVISION

CIVIL ACTION NO. 93-0544-L(S)

[Caption Omitted In Printing]

The deposition of WALID A. ABOU-JAOUDE, MD, taken pursuant to notice by the Defendant on March 31, 1995.

* * *

[p. 15] appropriate antibiotics to treat her pneumonias and her urinary trace infections.

She was pretty much stable for the time that I took care of her, and probably a couple weeks before her discharge, she was transferred to a step-down unit, which is the eighth floor. It's part of the ICU, but it's less intensive. And she progressed slowly. She made some improvement.

By system, neurologically, she was impaired when we first started the service. She had flaccid paralysis, the etiology we were not sure of. We had neurosurgeons following her as well. And we were not sure which way she's going to go, but neurologically she made some improvement in the time that I took care of her, that at the time of discharge, at least she was gazing a little bit and shrugging her shoulders, which she didn't have before.

Respiratory wise, she had multiple pneumonias, but again she responded to the antibiotics. And she was a



high-care patient in the sense that she required pulmonary toilet frequently and she – she required bronchoscopies, which is part of our pulmonary toilet protocol at the university. And again she progressed and we were [p. 16] able to get her off the ventilator, which was a big improvement at that time.

Gastrointestinal, I mean, she was feeding with the Dobbhoff tube placed in, but she wasn't eating, taking anything orally, and she was doing well with that. Prior to the time I took care of her, she had episodes of pancreatitis and upper GI bleeds, but all of these subsided by then. Neurologically, she had the indwelling Foley catheter, and if I recollect, she had about –

THE REPORTER: (Interrupting) She had –?

A About with yeast infection in the urine that responded to amphotericin, A-M-P-H-O-T-E-R-I-C-I-N, irrigation. And again she had urinary tract infections, again responded to antibiotics.

Otherwise, she was receiving rehabs and routine exercises. And we had speech therapy evaluate her. We had all the services in the hospital, as far as speech and rehab and respiratory work, with us in managing this patient. But as far as I recoilect, I mean, she went through the worst period of time prior to July.

[p. 17] Q Uh-huh.

A By July she was pretty much on a steady course of showing improvement.

Q Let me ask you, Doctor, you – you mentioned a couple of times that at the point when you began treating this patient in July, she appeared to be stable. Can you

define to – for me what – what you mean by appeared to be stable?

A By stable I mean that she was not acutely deteriorating or sick.

Q During that period of time, were there instances where she did have this recurring pneumonia?

A She did.

Q And during that same period of time, were there instances where she had this recurring urinary tract infection?

A She did.

Q Would those have been instances where – where the patient became less stable than – than she was at the beginning of the period of time when you began caring for her?

A Her pneumonias and her urinary tract infections did not put her in a position of instability. I mean, we were able to detect them

* * *

[p. 19] approach the social workers until we know that the patient is in a condition where she can be transferred, is stable enough to be transferred.

Q Doctor, have there been occasions while you were a resident at the University of Louisville Hospital where you had notified the social services department that a patient had become stable enough to transfer and requested that they begin looking for a placement, but



that at some point subsequent to that, the patient's condition deteriorated to such an extent that you had to delay transfer?

A Offhand, I cannot recollect a patient.

Q That's possible that that could happen, though, isn't it?

A It is possible.

Q Do you recall specifically in Wanda Johnson's case what involvement you had in discussions with the social services staff in their attempts to place Wanda?

A I recall that the social working – the social worker working at that time on the case, Nancy Fred, we approached her that Wanda is making progress and we need to look for a place for her to go following her discharge from the hospital.

[p. 20] And we made – at that time, when we approached Nancy, the pen – the patient was still on the ventilator, and we were looking for a nursing home that can provide ventilator equipment and nursing skills that can take care of the patients on ventilators, but – and that's what she started working on. But when Wanda made some improvement in her respiratory status and we were able to get her off the ventilator, we approached Nancy Fred again and we told her that now she's making progress, so we can look at a nursing home that has skilled care to take care of this patient now that she's off the ventilator.

So we approached her, it must have been a couple weeks before discharge, but the progress that we made

changed the course of her – the course of where she's going to, what nursing home.

Q You referred several times to we approached social workers. Were these social workers that you personally approached or was this someone else that was just on the surgical residency team that – that was talking to the social workers?

A Well, as a team on the trauma service, there's four people working on that team, and each [p. 21] has a different responsibility. There's a chief resident, chief resident, who is the last year surgical resident, and there is a third-year resident who takes care of the emergency room, and there's the second-year, or what we call the intensivist takes care of the ICU patients, and the first year. And depending on the patient you're taking care of, whether it's in the ICU or the floor, the patient respon – the – excuse me, the surgeon responsible of taking care of that patient approaches the social worker. But all of that is done in association with the chief resident.

Q All right. I guess my specific question, Doctor, is whether you personally had discussions with Nancy Fred about Wanda Johnson's discharge plan.

A I talked to her, to Nancy multiple times about the case, because I was familiar with it.

Q And – and the kinds of things that you discussed with Nancy included what?

A The first time I approached Nancy or –?

Q As best you can recall, the substance of any conversations that you had with Nancy about



* * *

[p. 29] of the conversation that you had with Nancy when you first learned that Crestview nursing home may be considering Wanda as a placement.

A Uh–huh. When Nancy Fred approached me with the fact that there is a nursing home, Crestview nursing home, that's willing to take this patient, we just wanted to make sure that it was skilled enough to take care of this patient. And like other nursing homes, they have a representative that comes to the hospital to visit the hospital, talk to the patient or the family and the nurses, and sometimes talk to us, as doctors, to see what's going on with her medical status, but that's been pretty much the routine. As far as Crestview, it was just part of the routine that we go through.

Q Do you recall ever talking to the representative from Crestview yourself?

A Approaching them first?

Q No. Well, no, speaking to them at any time prior to Wanda's discharge.

A No.

Q You mentioned that when Crestview was mentioned, you wanted to know that they could – they could adequately care for her, or properly?

A Right.

[p. 30] Q Do you recall what types of questions you asked Nancy Fred – let me strike that and ask you, prior to Nancy Fred mentioning Crestview nursing home in

Wanda Johnson's instance, do you know if you had ever dealt with Crestview nursing home before?

A I don't think so.

Q What kinds of questions did you ask Nancy Fred or what kinds of information did you request to make you feel comfortable that this facility could handle Wanda Johnson?

A First of all, I wanted to make sure it was a skilled nursing facility in the sense that they can provide the skilled care for this patient. We were definitely concerned about her condition, because she was – she was very sick when she first presented, and she was making progress, and we wanted to make sure that there is a facility or people that can attend to – to her needs and make sure that she continues to make progress.

Q Do you remember whether or not you asked Nancy Fred if she knew if Crestview nursing home had ever cared for a patient who needed the level of care that Wanda Johnson needed?

A I don't remember that.

* * *

[p. 34] Q Do you remember whether or not you ever asked Nancy Fred – strike that.

Were you concerned at all when you learned that the facility that Nancy had located was in Indianapolis, Indiana?

A No.



Q Did you feel that its geographic location was satisfactory to accommodate the needs of Wanda's family?

A As I told you, I never had any contact with the family.

Q Fair enough. Fair enough.

A Can I add something?

Q Certainly.

MR. THOMPSON: Sure.

A I mean, I never had any contact with the family because I could not reach the family. I could not reach them myself. I mean, we tried multiple times to get ahold of the family, but we could not get ahold of them, because I never saw anybody in the hospital to talk to and see, because I would like to get to know the family as well as the patient. So we depended a lot on the discharge planning to make the right calls to get ahold of the family so we know what's going on with her family.

* * *

[p. 37] her hospital – of her hospitalization at the University of Louisville?

A She was pretty much stable the – the whole time, or at least the last two weeks that we were taking care of her.

Q Did you actually dic – excuse me. Dictate the discharge summary for Wanda Johnson?

A Yes, I did.

Q All right. Let me –

MR. THOMPSON: (Interrupting) Before we get into that, can we take a two-minute break?

MR. MATTINGLY: Sure. (Whereupon a recess was had from approximately 2:22 to 2:42 p.m.)

Q Doctor, let me hand to you a document and let you take a look at that and ask you if you can identify that document.

MR. THOMPSON: Should I take these back?

MR. MATTINGLY: Pardon?

MR. THOMPSON: Should I take this back?

MR. MATTINGLY: No, I'm going to look at them again in a few minutes.

A I dictated that summary, but it's not

* * *

[p. 46] colon, and there are two dates there. Do you know what the two dates would have meant?

A No.

Q So you're – you're unsure of what the August 24, '92 date refers to on that discharge summary?

A I don't know what it – no.

Q Were you still in Louisville on August 24th?

A Yes, I was still on the trauma service.

Q Now, let's refer to what I think now we've marked as Number 4, which is – is the discharge summary that has been supplied by counsel for the hospital.



Could you explain to me how it came about that – that this discharge summary contained handwritten notes and – and what the purpose was for such an update?

A When I dictated the other one, that one (indicating), Number 3 –

Q (Interrupting) Uh-huh.

A (Continuing) – I had to update it before she went to – to the nursing home.

Q And why did you have to update it?

A Well, for one thing, I had to read it, [p. 47] make sure that they don't have any mistakes in it.

Q Uh-huh.

A And I filled in a couple blanks that were left, and then at the same time realized that we need to make sure that they know that she is on the T piece and that she's requiring the positive pressure ventilation.

Q DPs? What are DPs?

A On the T tube piece, T tube piece. It's just she's on a tracheostomy. She's on a 40 percent oxygen, which she was getting through the T tube. And if I recall correct, my chief resident asked me to specifically go over this and make sure that I mentioned that she is on that amount of oxygen and she's requiring that kind of pulmonary toilet as well –

Q (Interrupting) Okay.

A (Continuing) – before she got discharged.

Q All right. Any other changes that you felt necessary to make?

A Well, we started her on Bactrim, antibiotics, before she was discharged.

Q Okay. And why did you start her on Bactrim?

[p. 48] A Well, at that time, when she spiked a temperature –

Q (Interrupting) Uh-huh.

A (Continuing) – we worked her up for the fever, and which included – our workup included at that time a chest x-ray, urinalysis and – and a CBC, which is part of our routine workup for fevers. The urinalysis was cloudy and showed white blood cells in it that was elevated than previous analysis. She has a chronic indwelling catheter and she was predisposed to having colonizations and white blood cells in the urine, but that was elevated. At the same time, she had some bacteria that was not there in previous urinalysis. So we were suspecting a new urinary tract infection developing, and Bactrim is an excellent medication for that.

Q All right. Let me make sure I understand the time period that these – these events transpired or these changes in her condition occurred.

A Uh-huh.

Q You dictated a discharge summary on July 21st of 1992 that indicates that she was off antibiotics at that time, correct?

[p. 49] A Correct.

Q Now, is your testimony that from Jan – from July 21st, 1992 to July 24th, 1992, that – that she had spiked a temperature?



A She had spiked a temperature on the 23rd.

Q Okay.

A But during her hospital course, she was always spiking temperatures every so often. We – the cause of that, we – I mean, we would work her up, we would get the necessary urine specimen, x-rays and blood work to rule out any – any source of fever that we can identify and treat her, and that was just part of her course in the hospital, actually.

Q Would it be true that from July 21st, 1992 to July 24th, 1992, that Mrs. Johnson during that period of time developed a urinary tract infection?

A It could be possible.

Q You suspected that she had, did you not?

A Well, when she spiked the fever, we had – like I mentioned, we had the x-ray, we had blood – blood work and the urinalysis done. Our [p. 50] main concern was if she's developing a pneumonia or a urinary tract infection.

Q Uh-huh.

A And we wanted to work her up for – for both as a source of fever. And working up her urine – her UTI, urinary tract infection, we just send the urine and we – excuse me, and we know that that was cloudy, so we had a high suspicion. We didn't have the results of the urinalysis back then, but we could not wait 'til we get results back to treat the patient.

As far as her suspected pneumonia, we got a chest x-ray, and during her hospitalization she had some persistent infiltrate in the right upper lobe that was of concern to us, because it could – it – it would predispose her to having pneumonias, it would predispose her to having collapse, and that's why that night when she spiked, I remember I – I did a flexible bronchoscopy on her, which is part of our routine evaluation at University of Louisville, and we do the flexible bronchoscopy mostly to clear up the lungs from secretions, get rid of the plugs that can cause the lung to collapse.

And from reviewing my – my operative [p. 51] note, it looked like she had more secretions that we were able to suction with the bronchoscope more effectively than just with routine suctioning at that time, and that the – a specimen was taken and was sent to the lab just to make sure that she's not growing anything there that would turn into pneumonia. And the Gram's stain showed gram-positive cocci, which we don't get real worked up about it, because she has been in the intensive care unit for a while and she is prone to having colonizations from gram-positive coccis.

And – and empirically we just started her on Bactrim, because Bactrim has been used before to treat her bouts of pneumonia that she had and as well as it covers the urinary tract infection. So we felt that Bactrim would cover either one, not knowing exactly which one she had.

Q All right. And from the time that you took those – those cultures or – or specimens and submitted them to the lab, how long would it – would it have taken you to get the results back, generally?



A The urinalysis, we get it real quick. It takes sometimes an hour or two hours, but we get the results. We don't get the culture, because they [p. 52] have to plate it, it takes from 24 to 48 hours to get the specific organism that's growing. But at least we would get the urinalysis, microscopic urinalysis, which indicates how many white bloods cells, red blood cells, proteins, ketones, nitrites, leukocytes present in the urine, which would help us.

And then as far as the sputum culture or the bronchial specimen, the Gram's stain, it depends when you send it to the lab. Sometimes it can take overnight to get the results, but we usually get it within the next 12 hours or so. That would give us an indication of if there is anything nonspecific, either white blood cells or gram-positive rods, gram-negative rods, that's what we look for then. The culture can take from 24 to 48 hours, even more.

Q Do you recall whether you had received results back from any of these tests before Wanda Johnson was actually transferred from the hospital?

A Yes, we had the results of her urinalysis, and we had the results of the Gram's stain that was done on the bronchial specimen as well.

Q Do you recall when those tests were [p. 53] actually taken? Was it – was it the morning of the 24th or was it the day before it, if you recall?

A We did those when she spiked the temperature. That was the night of the 23rd.

Q And it's your recollection that you had those results back sometime the next morning?

A Yes. I documented them in my note the next morning.

Q Do you recall whether there was ever any discussion of holding Wanda for another couple of days until all of the results came back?

A Discussion among?

Q Among – between you and the social worker staff or between anyone else that you may have overheard.

A I presented the data to the chief.

Q Who is?

A Doctor Brian Harbrecht.

MR. THOMPSON: Spell the last name.

A H-A-R-B-R-E-C-H-T. He was the chief on the service at that time. And he was aware of those results before we even made the decision that morning to send her to the nursing home – nursing home.

Q Do you recall your specific

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**UNITED STATES DISTRICT COURT**
**WESTERN DISTRICT OF KENTUCKY**
**LOUISVILLE DIVISION**

**NO. C-93-0544L(CS)**

**JANE ROBERTS, as Guardian for WANDA Y. JOHNSON** **PLAINTIFF**

**v.**

**AFFIDAVIT**

**GALEN OF VIRGINIA, INC. d/b/a UNIVERSITY OF LOUISVILLE HOSPITAL** **DEFENDANT**

* * *

Affiant, David Richardson, M.D., being duly sworn, states as follows:

1. I was the attending physician responsible for supervising surgical residents on the Trauma Team who cared for Wanda Johnson at University of Louisville Hospital in July, 1992.

2. I have held the position of attending surgeon at the University of Louisville Hospital for 18 years.

3. During that time, I have never known a patient to be discharged because of financial reasons.

4. In July, 1992, I participated in Wanda Johnson's medical care.

5. Wanda Johnson's condition was sufficiently stable to warrant discharge on July 24, 1992.

6. I am certain that Wanda Johnson's financial situation was not a consideration in the medical decision to discharge her from University of Louisville Hospital.

Further Affiant Sayeth naught.

/s/ David Richardson, M.D.
DAVID RICHARDSON, M.D.

COMMONWEALTH OF KENTUCKY )
) ss.
County of Jefferson )

Subscribed and sworn to before me by David Richardson, M.D., this 10 day of March, 1995.

My Commission Expires: 2-21-96

/s/ Anne Cunningham
Notary Public
State at Large

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**UNITED STATES DISTRICT COURT**
**FOR THE**
**WESTERN DISTRICT OF KENTUCKY**
**LOUISVILLE DIVISION**

**NO. C-93-0544L-(CS)**

**JANE ROBERTS, as Guardian for WANDA Y. JOHNSON** — **PLAINTIFF**

**v.** **AFFIDAVIT**

**GALEN OF VIRGINIA, INC. d/b/a UNIVERSITY OF LOUISVILLE HOSPITAL** — **DEFENDANT**

* * *

Affiant, Walid Abou-Jaoude, M.D., being duly sworn, states as follows:

1. I was a second year surgical resident (intensivist) at University of Louisville Hospital in July, 1992.

2. I was responsible for the care of Wanda Johnson in July, 1992.

3. I wrote the order to discharge Wanda Johnson from University of Louisville Hospital in July, 1992.

4. In my opinion, Wanda Johnson's condition was medically stable on the day of her discharge from University of Louisville Hospital.

5. I was unaware of Wanda Johnson's financial status in July, 1992.

6. Wanda Johnson's financial status was not a factor in my decision to write the discharge order in July, 1992.

Further Affiant sayeth naught.

/s/ W. Abou-Jaoude
WALID ABOU-JAOUDE, M.D.

STATE OF PENNSYLVANIA )
) ss.
County of )

Subscribed and sworn to before me by Walid Abou-Jaoude, M.D., this 15 day of March March, 1995.

My Commission Expires: _____

/s/ Marcelle Didiere-West
Notary Public
State at Large

[Seal]

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**UNITED STATES DISTRICT COURT**
**FOR THE**
**WESTERN DISTRICT OF KENTUCKY**
**LOUISVILLE DIVISION**

**NO. C-93-0544L-(CS)**

**JANE ROBERTS, as Guardian for WANDA Y. JOHNSON** — **PLAINTIFF**

**v.** **AFFIDAVIT**

**GALEN OF VIRGINIA, INC. d/b/a UNIVERSITY OF LOUISVILLE HOSPITAL** — **DEFENDANT**

* * *

Affiant, Brian Harbrecht, M.D., being duly sworn, states as follows:

1. I was the Chief Resident at University of Louisville Hospital in July, 1992.

2. As Chief Resident, I was responsible for supervising intensivists at Humana Hospital University of Louisville.

3. I was consulted in and approved of Wanda Johnson's discharge from University of Louisville Hospital in July, 1992.

4. Wanda Johnson's condition was sufficiently stable to warrant her discharge, which was medically reasonable.

5. Physicians are typically not aware of a patient's payment source at the time of discharge.

6. Wanda Johnson's financial status played no part in the decision to discharge her from University of Louisville Hospital on July 24, 1992.

Further Affiant Sayeth naught.

/s/ Brian G. Harbrecht
BRIAN Harbrecht, M.D.

COMMONWEALTH OF KENTUCKY )
) ss.
County of Jefferson )

Subscribed and sworn to before me by Brian Harbrecht, M.D., this 7th day of April, 1995.

My Commission Expires: June 17, 1995

/s/ Kristina K. Micholovich
Notary Public
State at Large

[SEAL]

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**UNITED STATES DISTRICT COURT**
**WESTERN DISTRICT OF KENTUCKY**
**LOUISVILLE DIVISION**

**NO. C-93-0544L(CS)**

**JANE ROBERTS, as Guardian for WANDA Y. JOHNSON** — **PLAINTIFF**

**v.** **AFFIDAVIT**

**GALEN OF VIRGINIA, INC. d/b/a UNIVERSITY OF LOUSIVILLE HOSPITAL** — **DEFENDANT**

* * *

Affiant, Nancy Fred, MSW, being duly sworn states as follows:

1. I was the medical social worker at University of Louisville Hospital who was responsible for Wanda Johnson's discharge planning in July, 1992.

2. Pursuant to a physician's order, I made discharge arrangements for Ms. Johnson.

3. Because of a long-standing shortage of nursing home beds in Kentucky, Indiana nursing facilities were considered for Ms. Johnson.

4. At the time of Ms. Johnson's discharge, her fee status was "Medicaid pending."

5. I was under no pressure from any department of the hospital or physician to locate a nursing home placement for Wanda Johnson for financial reasons.

6. I have never received any pressure from any department of the hospital or any physician to locate a

nursing home placement for any patient for financial reasons.

7. I did not inform Joanna Nolte, the marketing representative from Crestview Health Care Center, that I was under pressure from any department of the hospital to locate a nursing home placement for Ms. Johnson for financial reasons.

8. As a medical social worker I have no authority to discharge a patient from the hospital.

9. The decision to discharge a patient is made by a physician based upon the patient's medical condition.

Further the affiant sayeth naught.

/s/ Nancy Fred, MSSW
Nancy Fred, MSW

COMMONWEALTH OF KENTUCKY )
:SS
COUNTY OF JEFFERSON )

Subscribed and sworn to before me by Nancy Fred, MSW, this 1st day of May, 1995.

My commission expires: July 26, 1996

/s/ Millicent A. Tanner
NOTARY PUBLIC
STATE AT LARGE,
KENTUCKY